

Credentialing and Recredentialing

CR 1: Credentialing Policies

The organization has a well-defined credentialing and recredentialing process for evaluating and selecting licensed independent practitioners to provide care to its members.

Intent

The organization has a rigorous process for evaluating and selecting practitioners.

Element A: Practitioner Credentialing Guidelines

The organization specifies:

1. The types of practitioners it credentials and recredentials.
2. The verification sources it uses.
3. The criteria for credentialing and recredentialing.
4. The process for making credentialing and recredentialing decisions.
5. The process for managing credentialing files that meet and do not meet the organization's established criteria.
6. The criteria for practitioner sanctions, complaints and other adverse events found during ongoing monitoring that need to be reviewed by the Credentialing Committee or other designated peer-review body.
7. The process for requiring that credentialing and recredentialing are conducted in a nondiscriminatory manner.
8. The process for notifying practitioners if information obtained during the organization's credentialing process varies substantially from the information they provided to the organization.
9. The process for notifying practitioners of the credentialing and recredentialing decision within 30 calendar days of the Credentialing Committee's decision.
10. The medical director or other designated physician's direct responsibility and participation in the credentialing program.
11. The process for securing the confidentiality of all information obtained in the credentialing process, except as otherwise provided by law.
12. The process for confirming that listings in practitioner directories and other materials for members are consistent with credentialing data, including education, training, board certification and specialty.
13. The process for documenting information and activities in credentialing files.

Summary of Changes

Clarifications

- Revised the look-back period for factors 6, 9, 13 and the verification of fellowship component of factor 12 to 12 months.
- Replaced "NCQA-accepted" with "NCQA-approved" in the factor 2 explanation.

- Added a sentence to the second paragraph under factor 12 to clarify an organization's policies and procedures state if the organization does not communicate with members under any circumstance.
- Changed "either" to "any" under the explanation of factor 13: Appropriate documentation.
- Clarified under "Automated credentialing system" that signature stamps are only acceptable if the practitioner is physically impaired.

Scoring	Met	Partially Met	Not Met
	The organization meets 11-13 factors	The organization meets 7-10 factors	The organization meets 0-6 factors

Data source Documented process

Scope of review **Product lines**

For Interim Surveys and First Surveys: This element applies to all product lines.

For Renewal Surveys: This element applies to the Medicaid product line only.

Documentation

NCQA reviews the organization's policies and procedures in effect throughout the look-back period.

Look-back period

For Interim Surveys: Prior to the survey date.

For First Surveys: 6 months.

For Renewal Surveys: 24 months; 12 months for factors 6, 9, 13 and the verification of fellowship component of factor 12.

Explanation

This element is a **structural requirement**. The organization must present its own documentation.

Practitioners within the scope of credentialing

Practitioners are within the scope of credentialing if all criteria listed below are met:

- Practitioners are licensed, certified or registered by the state to practice independently (without direction or supervision).
- Practitioners have an independent relationship with the organization.
 - An independent relationship exists when the organization directs members to see a specific practitioner or group of practitioners, including all practitioners members can select as primary care practitioners.
- Practitioners provide care to members under the organization's medical benefits.

The listed criteria apply to practitioners in the following settings:

- Individual or group practices.
- Locum tenens:

- Provisional credentialing is required if these practitioners work less than 60 calendar days.
- Full credentialing is required if these practitioners work 60 calendar days or more.
- Facilities.
- Telemedicine.
- Rental networks:
 - That are part of the organization’s primary network, and the organization has members who reside in the rental network area.
 - Specifically for out-of-area care, and members may see only those practitioners, or are given an incentive to see rental network practitioners.
- PPO network:
 - If an organization contracts with a PPO network to provide health services to members who need care outside its service area, and if it encourages members to obtain care from that network, when they are outside the network, NCQA considers this to be an independent relationship if:
 - Information about the network is included in member materials or on an ID card that directs members to the network (e.g., network name, phone number, logo), **or**
 - There are incentives for members to see the PPO’s practitioners.

In this type of contractual arrangement, the organization must credential the practitioners or delegate credentialing to the PPO network.

Factor 1: Types of practitioners

Credentialing policies and procedures include the following types of practitioners.

- Medical practitioners:
 - Medical doctors.
 - Oral surgeons.
 - Chiropractors.
 - Osteopaths.
 - Podiatrists.
 - Nurse practitioners.
 - Other medical practitioners who may be within the scope of credentialing (e.g., physician assistant).
 - NCQA does not include these practitioners in the credentialing file review.
- Behavioral healthcare practitioners:
 - Psychiatrists and other physicians.
 - Addiction medicine specialists.
 - Doctoral or master’s-level psychologists.
 - Master’s-level clinical social workers.

- Master’s-level clinical nurse specialists or psychiatric nurse practitioners.
- Other behavioral healthcare specialists who may be within the scope of credentialing (e.g., licensed professional counselor).

Factor 2: Verification sources

Credentialing policies and procedures describe the sources the organization uses to verify credentialing information. The organization uses any of the following sources to verify credentials:

- The primary source (or its website).
- A contracted agent of the primary source (or its website).
 - The organization obtains documentation indicating a contractual relationship between the primary source and the agent that entitles the agent to verify credentials on behalf of the primary source.
- An NCQA-approved source listed for the credential (or its website).

Factors 3, 4: Decision-making criteria and process

The organization:

- Credentials practitioners before they provide care to members.
- Has a process for making credentialing decisions, and defines the criteria it requires to reach a credentialing decision.
 - Credentialing criteria are designed to assess a practitioner’s ability to deliver care.
 - Credentialing criteria are reviewed and approved by the Credentialing Committee, medical director or other designated peer review body.
- Makes final determination regarding which practitioners may participate in its network, based on its credentialing decision-making criteria.

Factor 5: Managing files that meet/do not meet criteria

Credentialing policies and procedures describe the process for determining and approving files that meet criteria (i.e., clean files) and that do not meet criteria. The organization may present all practitioner files to the Credentialing Committee, or may designate approval authority of clean files to the medical director or to an equally qualified practitioner.

Factor 6: Criteria for ongoing monitoring notifications to the Credentialing Committee

Credentialing policies and procedures describe the types of practitioner sanctions, complaints and other adverse events found during ongoing monitoring that need to be reviewed by the Credentialing Committee or other designated peer-review body. The policies and procedures describe the process for reporting the information to the Credentialing Committee or other designated peer-review body at the next meeting after the identified occurrence, as required in CR 5.

Factor 7: Nondiscriminatory credentialing and recredentialing

Credentialing policies and procedures:

- State that the organization does not base credentialing decisions on an applicant's race, ethnic/national identity, gender, age, sexual orientation or patient's insurance coverage (e.g., Medicaid) in which the practitioner specializes.
- Specify the process for preventing discriminatory practices.
 - Proactive steps to protect against discrimination in credentialing and recredentialing processes.
- Specify how the organization annually monitors the credentialing and recredentialing processes for discriminatory practices.
 - Track and identifies discrimination in credentialing and recredentialing processes.

Factor 8: Discrepancies in credentialing information

Credentialing policies and procedures describe the organization's process for notifying practitioners when credentialing information obtained from other sources during credentialing varies from that provided by the practitioner in the credentialing application.

Factor 9: Notification of decisions

Credentialing policies and procedures specify that the organization's time frame for notifying applicants of initial credentialing decisions and recredentialing denials does not exceed 30 calendar days from the credentialing decision. The organization is not required to notify practitioners regarding recredentialing approvals.

Factor 10: Participation of a medical director or designated physician

Credentialing policies and procedures describe the medical director or other designated physician's overall responsibility and participation in the credentialing process.

Factor 11: Ensuring confidentiality

Credentialing policies and procedures describe the organization's process for ensuring confidentiality of the information collected during the credentialing process and the procedures it uses to keep this information confidential.

Factor 12: Practitioner directories and member materials

Credentialing policies and procedures describe the organization's process for ensuring that information provided in member materials and practitioner directories is consistent with the information obtained during the credentialing process.

Credentialing policies and procedures specifies that the organization verifies completion of fellowship if the practitioner's fellowship program or completion of fellowship is communicated to members (e.g., in a directory, newsletter; by member services staff). If an organization does not communicate fellowship to

members, credentialing policies and procedures state that the organization does not communicate practitioner fellowship to members.

Note: *Verification of fellowship does not meet the intent of verifying the highest level of education and training.*

Factor 13: Appropriate documentation

Credentialing policies and procedures define the organization's process for documenting information and activities in credentialing files. The organization documents verification in the credentialing files using any of the following methods, or a combination:

- Credentialing documents signed (or initialed) and dated by the verifier.
- A checklist that includes, for each verification:
 - The source used.
 - The date of verification.
 - The signature or initials of the person who verified the information.
 - Typed initials are only acceptable if there is a unique electronic signature or identifier on the checklist.
 - The report date, if applicable.
- A checklist with a single signature and a date for all verifications that includes a statement confirming that the signatory verified all credentials on that date and that includes for each verification:
 - The source used.
 - The report date, if applicable.

Exception

This element is NA for Renewal Surveys for the commercial, Medicare and Exchange product lines.

Related information

Verification from a report. NCQA uses the date generated by the source when the information is retrieved. If the source report does not generate a date, NCQA uses the date noted in the credentialing file by the organization staff who verified the credentials. Staff who verified the credentials must sign or initial the verification.

Automated credentialing system. The organization may use an electronic signature or unique electronic identifier of staff to document verification if it can demonstrate that the electronic signature or unique identifier can only be entered by the signatory. The organization provides its security and login policies and procedures to confirm the unique identifier and that the signature can only be entered by the signatory. The system must identify the individual verifying the information, the date of verification, the source and the report date, if applicable.

Faxed, digital, electronic, scanned or photocopied signatures are acceptable. Signature stamps are not acceptable, unless the practitioner is physically impaired.

Use of software for data collection. NCQA does not consider it delegation if the organization uses another entity's software to collect credentialing information, unless the entity also reviews the credentialing information on behalf of the organization.

Use of web crawlers. The organization may use web crawlers to verify credentialing information from NCQA-approved sources. A "web crawler" is software that retrieves information directly from a primary or NCQA-approved source website (e.g., the state licensing or certification agency). The organization provides documentation that the web crawler collects information only from NCQA-approved sources, and documents that staff reviewed the credentialing information.

Use of application program interface (API). The organization may use an API to access data from a primary or NCQA-approved source, and must provide documentation that the API collects information only from primary or NCQA-approved sources. Organizations must still meet the appropriate documentation requirements in factor 13, including documentation that the organization's staff reviewed the information.

Note: *The use of another entity's software to collect credentialing information is not considered delegation unless the entity also reviews the information on the organization's behalf.*

Provisional credentialing. If the organization decides to provisionally credential practitioners, it:

- Has a process for one-time provisional credentialing of practitioners applying to its network for the first time.
- Verifies the following within the required time limits:
 - A current, valid license to practice (CR 3, Element A, factor 1).
 - The past 5 years of malpractice claims or settlements from the malpractice carrier, or the results of the National Practitioner Data Bank (NPDB) query (CR 3, Element A, factor 6).
 - A current and signed application with attestation (CR 3, Element C, factors 1–6).
- Follows the same process for presenting provisional credentialing files to the Credentialing Committee or medical director as it does for its regular credentialing process.
- Does not perform provisional credentialing for practitioners who were credentialed by a delegate on behalf of the organization.
- Does not hold practitioners in provisional status for longer than 60 calendar days.
- Does not list provisionally credentialed practitioners in the directory.
- Does not allow practitioners to deliver care prior to completion of provisional credentialing.

Practitioners who do not need to be credentialed.

- Practitioners who practice exclusively in an inpatient setting and provide care for organization members only because members are directed to the hospital or another inpatient setting.

- Practitioners who practice exclusively in free-standing facilities and provide care for organization members only because members are directed to the facility.
- Pharmacists who work for a pharmacy benefits management (PBM) organization to which the organization delegates utilization management (UM) functions.
- Covering practitioners (e.g., locum tenens) who do not have an independent relationship with the organization are outside NCQA's scope of credentialing.
- Practitioners who do not provide care for members (e.g., board-certified consultants who may provide a professional opinion to the treating practitioner).
- Rental network practitioners who provide out-of-area care only, and members are not required or given an incentive to seek care from them.

Practitioner termination and reinstatement. The organization:

- Initially credentials a practitioner again if the break in network participation is more than 30 calendar days.
 - Re-verifies credentials that are no longer within verification time limits.
 - Re-verifies credentials that will not be in effect when the Credentialing Committee or medical director makes the credentialing decision.

Examples

Electronic signature software applications

- Adobe Sign.
- DocuSign.

Factor 7: Nondiscriminatory credentialing and recredentialing

The organization monitors credentialing decisions to prevent discrimination. Monitoring includes, but is not limited to:

- Maintaining a heterogeneous Credentialing Committee membership and requiring for those responsible for credentialing decisions to sign a statement affirming that they do not discriminate.
- Periodic audits of credentialing files (in-process, denied and approved files) that suggest potential discriminatory practice in selecting practitioners.
- Annual audits of practitioner complaints for evidence of alleged discrimination.

Element B: Practitioner Rights

The organization notifies practitioners about their right to:

1. Review information submitted to support their credentialing application.
2. Correct erroneous information.
3. Receive the status of their credentialing or recredentialing application, upon request.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 2-3 factors	No scoring option	The organization meets 0-1 factors
Data source	Documented process, Materials		
Scope of review	Product lines <i>For Interim Surveys and First Surveys: This element applies to all product lines.</i> <i>For Renewal Surveys: This element applies to the Medicaid product line only.</i> Documentation <i>For All Surveys:</i> NCQA reviews the organization's policies and procedures for all three factors. <i>For First Surveys and Renewal Surveys:</i> NCQA also reviews three materials sent to practitioners throughout the look-back period, or reviews all materials if the organization has fewer than three.		
Look-back period	<i>For Interim Surveys:</i> Prior to the survey date. <i>For First Surveys:</i> 6 months. <i>For Renewal Surveys:</i> 24 months.		
Explanation	This element is a structural requirement. The organization must present its own documentation. Factor 1: Review information The organization notifies practitioners of their right to review information obtained from outside sources (e.g., malpractice insurance carriers, state licensing boards) to support their credentialing application. The organization is not required to make available: • References. • Recommendations. • Peer-review protected information.		

Factor 2: Correct erroneous information from other sources

The organization notifies practitioners of their right to correct erroneous information, and:

- The time frame for making corrections.
- The format for submitting corrections.
- Where to submit corrections.

The organization is not required to reveal the source of information that was not obtained to meet verification requirements or if federal or state law prohibits disclosure.

The organization documents receipt of corrected information in the practitioner's credentialing file.

Factor 3: Application status

The organization notifies practitioners of:

- Their right to be informed of the status of their application, upon request.
- The information it is allowed to share with practitioners.
- Its process for responding to requests for application status.

Exception

This element is NA for Renewal Surveys for the commercial, Medicare and Exchange product lines.

Examples**Avenues for notification**

- Application.
- Contract.
- Provider manual.
- Other information distributed to practitioners.
- Website.
- Letter to practitioners.

Factor 2: Areas where information may vary

- Actions on a license.
- Malpractice claims history.
- Board certification.

CR 2: Credentialing Committee

The organization designates a Credentialing Committee that uses a peer-review process to make recommendations regarding credentialing decisions.

Intent

The organization obtains meaningful advice and expertise from participating practitioners when it makes credentialing decisions.

Element A: Credentialing Committee

The organization's Credentialing Committee:

1. Uses participating practitioners to provide advice and expertise for credentialing decisions.
2. Reviews credentials for practitioners who do not meet established thresholds.
3. Ensures that files that meet established criteria are reviewed and approved by a medical director, designated physician or the Credentialing Committee.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 2-3 factors	No scoring option	The organization meets 0-1 factors
Data source	Documented process, Reports		
Scope of review	Product lines <i>For Interim Surveys, First Surveys and Renewal Surveys: This element applies to all product lines.</i>		
	Documentation <i>For Interim Surveys:</i> NCQA reviews Credentialing Committee minutes from three different meetings. If three meeting minutes are not available for review, NCQA reviews the minutes from the look-back period. If the committee has not met, NCQA reviews the committee charter, and reviews a timeline for operationalizing the committee. <i>For First Surveys and Renewal Surveys:</i> NCQA reviews Credentialing Committee meeting minutes from three different meetings within the look-back period. If the required meeting minutes are not available for review, NCQA reviews the minutes from the look-back period.		
Look-back period	<i>For Interim Surveys:</i> Prior to the survey date. <i>For First Surveys:</i> 6 months. <i>For Renewal Surveys:</i> 24 months.		

Explanation **Factor 1: Participating practitioners**

The Credentialing Committee is a peer-review body with members from the types of practitioners participating in the organization's network.

The organization may have separate review bodies for each practitioner type (e.g., physician, oral surgeon, psychologist), specialty or multidisciplinary committee, with representation from various specialties.

If the organization is part of a regional or national organization, a regional or national Credentialing Committee that meets the criterion may serve as the peer review committee for the local organization.

Note: *Participating practitioners are external to the organization and are part of the organization's network.*

Factor 2: Committee review

The Credentialing Committee:

- Reviews the credentials of practitioners who do not meet the organization's criteria for participation in the network.
- Gives thoughtful consideration to credentialing information.
- Documents discussions about credentialing in meeting minutes.

Meetings and decisions may take place in real-time, virtual meetings (i.e., through video conference or web conference with audio), but may not be conducted only through email.

Factor 3: Review of files that meet established thresholds

For files that meet the organization's credentialing criteria, the organization:

- Submits all practitioner files to the Credentialing Committee for review, **or**
- Has a process for medical director or qualified physician review and approve clean files.
 - Evidence of review and approval is a handwritten signature, handwritten initials or unique electronic identifier, if the organization has appropriate controls for ensuring that only the designated medical director or qualified physician can enter the electronic signature.
 - An individual signature is not required in each practitioner file if there is one report with a signature that lists all required credentials for all practitioners with clean files.
 - Clean files that meet the organization's established criteria may be reviewed by email.

NCQA scores this factor "Yes" if the organization presents all files (including clean files) to the Credentialing Committee.

Exceptions

None.

Related information

Assessment of timeliness. NCQA considers a practitioner to be credentialed as of the Credentialing Committee or date of the medical director's decision, and uses this date to assess timeliness in the file review elements, even if a review board or governing body reviews decisions made by the Credentialing Committee or medical director.

Providing care to members. The organization does not permit uncredentialed practitioners to provide care to members.

Some states require reimbursement of practitioners back to the date the application, for members seen during the credentialing process period, if the organization subsequently decides to credential the practitioner. Such retroactive payment is outside the scope of NCQA's credentialing requirement.

Examples

None.

CR 3: Credentialing Verification

The organization verifies credentialing information through primary sources, unless otherwise indicated.

Intent

The organization conducts timely verification of information to ensure that practitioners have the legal authority and relevant training and experience to provide quality care.

Element A: Verification of Credentials

The organization verifies that the following are within the prescribed time limits:

1. A current and valid license to practice.
2. A valid DEA or CDS certificate, if applicable.
3. Education and training as specified in the explanation.
4. Board certification status, if applicable.
5. Work history.
6. A history of professional liability claims that resulted in settlement or judgment paid on behalf of the practitioner.

Summary of Changes

Clarifications

- Added “Future dates of completion” subhead and a Note to the factor 3 explanation to clarify that NCQA does not require graduation or certificate date.
- Revised the Note in the factor 4 explanation to clarify that “other health care professionals” are defined in CR 1, Element A.
- Revised the language under the second bullet under “Other acceptable verification sources for physicians (MD, DO)” to clarify that the board or registry that performs primary source verification is an acceptable source.

Scoring	Met	Partially Met	Not Met
	High (90-100%) on file review for at least 4 factors and medium (60-89%) on file review for any remaining factors	High (90-100%) or medium (60-89%) on file review for 6 factors	Low (0-59%) on file review for any factor

Data source Records or files

Scope of review **Product lines**

For First Surveys and Renewal Surveys: This element applies to all product lines.

Documentation

For factors 1–6, NCQA reviews verification of credentials within a random sample of up to 40 initial credentialing files and up to 40 recredentialing files for practitioners that were due for recredentialing during the look-back period.

For factor 2, practitioners who are DEA- or CDS- eligible who do not have a DEA or CDS certificate, NCQA also reviews the credentialing file for documentation of the practitioner's lack of DEA/CDS certificate and the name of a designated alternate prescriber.

Look-back period

For First Surveys: 6 months.

For Renewal Surveys: 36 months.

Explanation

THIS IS A MUST-PASS ELEMENT.

This element applies to:

- Practitioners in the scope of credentialing, as defined in *CR 1: Credentialing Policies*, Element A.
- Initial credentialing and recredentialing files, unless an exception noted below applies.

For factors with verification time limits, NCQA counts back from the decision date to the verification date to assess timeliness of verification.

All credentials must be current at the time of the Credentialing Committee decision.

Appropriate documentation

Each file contains evidence of verification from a NCQA-listed source and review by organization staff. Refer to CR 1, Element A, factor 13.

Dispute of file review results

NCQA conducts onsite file review in the presence of the organization's staff and works with the organization to resolve any disputes during the onsite survey. An organization that is unable to resolve a dispute with the survey team must contact NCQA before the onsite survey is complete. File review results may not be disputed or appealed once the onsite survey is complete.

Factor 1: Licensure

Verification time limit: 120 calendar days.

Note: *The 120-calendar-day verification time limit applies to files processed by the organization or its delegate(s) on or after July 1, 2025. Files processed before July 1, 2025, are scored against the previous verification time limit requirement of 180 calendar days.*

The organization verifies that the practitioner has a valid and current license to practice at the time of the credentialing decision, in all states where the practitioner provides care to members. The organization must verify license directly from state licensing or certification agency (or its website).

Factor 2: DEA or CDS certificates

Verification time limit: Prior to the credentialing decision.

This factor applies to practitioners who are qualified to write prescriptions. The organization verifies that the practitioner's Drug Enforcement Administration (DEA) or Controlled Dangerous Substances (CDS) certificate is valid and current in each state where the practitioner provides care to members.

Acceptable verification sources:

- DEA or CDS agency.
- DEA or CDS certificate, or a photocopy of the certificate.
- Documented visual inspection of the original DEA or CDS certificate.
- Confirmation from the American Medical Association (AMA) Physician Masterfile (DEA only).
- American Osteopathic Association Official Osteopathic Physician Profile Report or Physician Master File (DEA only).
- Confirmation from the state pharmaceutical licensing agency, where applicable.

Pending DEA certificates

The organization may credential a practitioner whose DEA certificate is pending if it has a documented process for allowing a practitioner with a valid DEA certificate to write all prescriptions requiring a DEA number for the prescribing practitioner whose DEA is pending until the practitioner has a valid DEA certificate.

DEA- and CDS-eligible practitioners who do not have a certificate

The organization verifies that all DEA- and CDS-eligible practitioners who do not have a valid DEA/CDS certificate, and for whom prescribing controlled substance is in the scope of their practice, have in place a designated practitioner to write prescriptions on their behalf. The organization documents the practitioner's lack of DEA/CDS certificate in the credentialing file and obtains the name of a designated alternate prescriber from the practitioner. If the alternate prescriber is a practice rather than an individual, the file may include the practice name. The organization is not required to arrange an alternate prescriber.

If the practitioner states in writing that they do not prescribe controlled substances and that in their professional judgment, the patients receiving their care do not require controlled substances, they are therefore not required to have a DEA/CDS certificate, but must describe their process for handling instances when a patient requires a controlled substance. The organization includes the practitioner's statement and process description in the credentialing file.

Factor 3: Education and training

Verification time limit: Prior to the credentialing decision.

The organization verifies the highest of the following three levels of education and training obtained by the practitioner as appropriate:

1. Board certification.
2. Residency.
3. Graduation from medical or professional school.

Note: *Verification of fellowship does not meet the intent of verifying the highest level of education and training.*

The organization uses any of the following to verify education and training:

- The primary source.
- The state licensing agency, specialty board or registry, if it performs primary source verification.
 - The organization:
 - Obtains written confirmation at least annually from the state licensing agency, specialty licensing agency, specialty board or registry that primary-source verifies education and training information, **or**
 - Provides a printed, dated screenshot of the state licensing agency, specialty board or registry website displaying the statement that it performs primary source verification of practitioner education and training information, **or**
 - Provides evidence of a state statute requiring the licensing agency, specialty board or registry to obtain verification of education and training directly from the institution.
- Sealed transcripts, if the organization provides evidence that it inspected the contents of the envelope and confirmed that the practitioner completed (graduated from) the appropriate training program.

Future dates of completion

Future dates of program completion do not meet the intent of this factor.

Note: *If a program is completed earlier than the issue date of the certificate of graduation or residency program, the organization may provide a letter from the primary source (e.g., program director) that the practitioner completed all program requirements, as evidence for factor 3.*

Other acceptable verification sources for physicians (MD, DO)

Board certification

- For physicians (MD, DO):
 - ABMS or its member boards or an official ABMS Display Agent, where a dated certificate of primary-source authenticity has been provided.

Note: The ABMS “Is Your Doctor Board Certified,” accessible through the ABMS website, is intended for consumer reference only and is not an acceptable source for verifying board certification.
 - AMA Physician Masterfile.
 - AOA Official Osteopathic Physician Profile Report or AOA Physician Master File.
 - Boards in the United States that are not members of the ABMS or AOA (e.g., National Board of Physicians and Surgeons), if:
 - The organization’s policies and procedures document which specialty boards it accepts, **and**

- If the organization obtains annual written confirmation from the board that the board performs primary source verification of completion of education and training.
- For other health care professionals:
 - Board or registry that performs primary source verification of board status, if the organization obtains annual written confirmation that the board or registry performs primary source verification of board certification status.

Organizations may use expired board certification to meet verification of education and training because primary-source verified education and training information do not change with expiration of board certification.

Graduation from medical school

- AMA Physician Masterfile.
- American Osteopathic Association (AOA) Official Osteopathic Physician Profile Report or AOA Physician Master File.
- Educational Commission for Foreign Medical Graduates (ECFMG) for international medical graduates licensed after 1986.

Completion of residency training

- AMA Physician Masterfile.
- AOA Official Osteopathic Physician Profile Report or AOA Physician Master File.
- FCVS for closed residency programs.

NCQA only recognizes residency programs accredited by the Accreditation Council for Graduate Medical Education (ACGME), the Accreditation Council for Graduate Medical Education—International, the American Osteopathic Association (in the United States), the College of Family Physicians of Canada or the Royal College of Physicians and Surgeons of Canada.

Factor 4: Board certification status

Verification time limit: 120 calendar days.

Note: *The 120-calendar-day verification time limit applies to files processed by the organization or its delegate(s) on or after July 1, 2025. Files processed before July 1, 2025, are scored against the previous verification time limit requirement of 180 calendar days.*

Although NCQA does not require board certification, the organization must verify current certification status of practitioners who state they are board certified.

The organization documents the expiration date of the board certification in the credentialing file. If a practitioner has a certification that does not expire (e.g., a lifetime certification status), the organization verifies that board certification is current, and documents the date of verification. If the expiration date is not provided, the organization may leave the expiration date blank in the practitioner file.

Verification sources

The organization uses any of the following to verify board certification:

- For all practitioner types:
 - The primary source (appropriate specialty board).
 - The state licensing agency, if it primary-source verifies board certification.
- For physicians (MD, DO), the sources listed under Factor 3: Education and training.

Note: Verification of board certification does not apply to nurse practitioners or other health care professionals who may be within the scope of credentialing (as defined in CR 1, Element A), unless the organization communicates board certification to members.

Factor 5: Work history

Verification time limit: 180 calendar days.

Note: The 180-calendar-day verification time limit applies to files processed by the organization or its delegate(s) on or after July 1, 2025. Files processed before July 1, 2025, are scored against the previous verification time limit requirement of 365 calendar days.

Employment dates. The organization obtains a minimum of the most recent 5 years of work history as a health professional through the practitioner's application or CV. If the practitioner has fewer than 5 years of work history, the time frame starts at the initial licensure date.

The application or CV includes the beginning and ending month and year for each position of employment experience, unless the practitioner has had continuous employment for 5 years or more with no gap. In this case, providing the year meets the intent of this factor.

Gaps in work history. The organization documents its review of the practitioner's work history and any gaps on the application, CV, checklist or other identified documentation methods (i.e., signature or initials of staff who reviewed the history and the date of review).

- If a gap in employment exceeds 6 months, the practitioner clarifies the gap verbally or in writing. The organization documents a verbal clarification or includes the written notice in the practitioner's credentialing file.
- If the gap in employment exceeds 1 year, the practitioner clarifies the gap in writing, and the organization documents its review.

Factor 6: Malpractice history

Verification time limit: 120 calendar days.

Note: The 120-calendar-day verification time limit applies to files processed by the organization or its delegate(s) on or after July 1, 2025. Files processed before July 1, 2025, are scored against the previous verification time limit requirement of 180 calendar days.

The organization obtains confirmation of the past 5 years of malpractice settlements from the malpractice carrier, or queries the National Practitioner

Databank (NPDB). The 5-year period may include residency or fellowship years. The organization is not required to obtain confirmation from the carrier for practitioners who had a hospital insurance policy during a residency or fellowship.

Exceptions

This element is NA for Interim Surveys.

Factors 3 and 5 are NA for recredentialing files.

Factor 4 is NA if the practitioner is:

- Not board certified, **or**
- A board-certified nurse practitioner or other health care professional, but the organization does not communicate board certification to members.

Related information

Query results (factor 6). The organization is not required to share query results with NCQA. NCQA accepts documentation of the query and of the organization's receipt of the information.

Examples

Factor 2: DEA- and CDS-eligible practitioners who do not have a certificate

Practitioner's statement: I do not prescribe controlled substances for my patients. If I determine that a patient may require a controlled substance, I refer the patient to their primary care practitioner or to another practitioner for evaluation and management.

Element B: Sanction and Exclusion Information

The organization verifies the following information at credentialing:

1. State sanctions, restrictions on licensure and limitations on scope of practice.
2. Medicare and Medicaid sanctions.
3. Medicare and Medicaid exclusions.

Summary of Changes

Clarifications

- Revised the look-back period for factor 3 to 12 months.

Scoring

Met	Partially Met	Not Met
High (90-100%) on file review for at least 2 factors and medium (60-89%) on file review for the remaining factor	High (90-100%) or medium (60-89%) for 3 factors	Low (0-59%) on file review for any factor

Data source Records or files

Scope of review	Product lines <i>For First Surveys and Renewal Surveys: This element applies to all product lines.</i> Documentation NCQA reviews verification of sanction and exclusion information within a random sample of up to 40 initial credentialing files and up to 40 recredentialing files for practitioners that were due for recredentialing during the look-back period.
Look-back period	<i>For First Surveys: 6 months.</i> <i>For Renewal Surveys: 36 months, 12 months for factor 3.</i>
Explanation	THIS IS A MUST-PASS ELEMENT. This element applies to: • Practitioners in the scope of credentialing, as defined in <i>CR 1: Credentialing Policies</i>, Element A. • Initial credentialing and recredentialing files, unless an exception noted below applies. The organization verifies sanction and exclusion information (from factors 1–3) for all product lines. Dispute of file review results NCQA conducts onsite file review in the presence of the organization's staff, and works with the organization to resolve any disputes during the onsite survey. An organization that is unable to resolve a dispute with the survey team must contact NCQA before the onsite survey is complete. File review results may not be disputed or appealed once the onsite survey is complete. Appropriate documentation <i>Verification time limit: 120 calendar days.</i> Note: <i>The 120-calendar-day verification time limit applies to files processed by the organization or its delegate(s) on or after July 1, 2025. Files processed before July 1, 2025, are scored against the previous verification time limit requirement of 180 calendar days.</i> NCQA counts back from the decision date to the verification date to assess timeliness of verification. Each practitioner file contains evidence of verification of sanction and exclusion information from a NCQA-listed source and review by organization staff. Refer to CR 1, Element A, factor 13. Factor 1: Scope of review for sanctions and limitations on licensure The organization verifies state sanctions, restrictions on licensure and limitations on scope of practice in all states where the practitioner provides or has provided care to members for the most recent 5-year period available. If practitioners were licensed in more than one state in the most recent 5-year period, the query must include all states in which they provided care. The

organization may obtain verification from the NPDB for all practitioner types listed below.

The following sources may be used for verification:

- Physicians:
 - Appropriate state agencies.
 - Federation of State Medical Boards (FSMB).
- Chiropractors:
 - State Board of Chiropractic Examiners.
 - Federation of Chiropractic Licensing Boards' Chiropractic Information Network-Board Action Databank (CIN-BAD).
- Oral surgeons:
 - State Board of Dental Examiners or State Medical Board.
- Podiatrists:
 - State Board of Podiatric Examiners.
 - Federation of Podiatric Medical Boards.
- Other nonphysician health care professionals:
 - State licensure or certification board.
 - Appropriate state agency.

Factor 2: Sources for Medicare/Medicaid sanctions

The organization obtains Medicaid sanction information from the State Medicaid agency and from one of the following additional sources:

- AMA Physician Master File.
- FSMB.
- NPDB.
- SAM.gov.

The organization obtains Medicare sanction information from any of the following sources:

- AMA Physician Master File.
- FSMB.
- NPDB.
- SAM.gov.

Factor 3: Sources for Medicare/Medicaid exclusions

The organization obtains Medicaid exclusion information from the State Medicaid agency and from one of the following additional sources:

- List of Excluded Individuals and Entities, maintained by OIG and available over the internet, **or**
- NPDB.

The organization obtains Medicare exclusion information from any of the following sources:

- Medicare Exclusion Database.
- List of Excluded Individuals and Entities, maintained by OIG and available over the internet.
- NPDB.

Exception

This element is NA for Interim Surveys.

Related information

Use of verifications in CR 5: Ongoing Monitoring and Interventions. The organization may use ongoing monitoring of sanction information from CR 5, Element A, factors 1 and 2 to meet CR 3, Element B if the information is no more than 120 calendar-days old at the time of the credentialing decision.

Query results. The organization is not required to share query results with NCQA. NCQA accepts documentation of the query and of the organization's receipt of the information.

Examples None.

Element C: Credentialing Application

Applications for credentialing include the following:

1. Reasons for inability to perform the essential functions of the position.
2. Lack of present illegal drug use.
3. History of loss of license and felony convictions.
4. History of loss or limitation of privileges or disciplinary actions.
5. Current malpractice insurance coverage.
6. Practitioner race, ethnicity and language.
7. Current and signed attestation confirming the correctness and completeness of the application.

Summary of Changes

Clarifications

- Added a second paragraph to the scope of review to clarify that factor 6 applies to applications processed on or after July 1, 2025.
- Revised the look-back period to 12 months for factor 6.
- Rearranged and revised the last three paragraphs of the factor 5 explanation.
- Added "This meets the NCQA required time frame" as the second sentence in the Note in the factor 7 explanation.
- Added an exception to clarify that applications processed before July 1, 2025, are NA for factor 6.

Scoring	Met	Partially Met	Not Met
	High (90-100%) on file review for at least 4 factors and medium (60-89%) on file review for the remaining factors	High (90-100%) or medium (60-89%) on file review for 7 factors	Low (0-59%) on file review for any factor
Data source	Records or files		
Scope of review	Product lines <i>For First Surveys and Renewal Surveys: This element applies to all product lines.</i>		
	Documentation NCQA reviews application and attestation within a random sample of up to 40 initial credentialing files and up to 40 recredentialing files for practitioners that were due for recredentialing during the look-back period. <i>For factor 6:</i> • Applications processed before July 1, 2025 will not be scored on this factor. • Applications processed on or after July 1, 2025, will be scored on this factor, but will not be held to the 6-month look-back period until surveys on or after January 1, 2026.		
Look-back period	<i>For First Surveys: 6 months.</i> <i>For Renewal Surveys: 36 months for factor 1–5 and 7; 12 months for factor 6.</i>		
Explanation	THIS IS A MUST-PASS ELEMENT.		
	This element applies to:		
	• Practitioners in the scope of credentialing as defined in CR 1, Element A. • Initial credentialing and recredentialing files, unless an exception applies.		
	NCQA counts back from the decision date to the verification date to assess timeliness of credentialing and recredentialing decisions.		
	Dispute of file review results		
	NCQA conducts onsite file review in the presence of the organization's staff and works with the organization to resolve any disputes during the onsite survey. An organization that is unable to resolve a dispute with the survey team must contact NCQA before the onsite survey is complete. File review results may not be disputed or appealed once the onsite survey is complete.		
	Appropriate documentation		
	<i>Attestation verification time limit: 180 calendar days.</i>		
	Note: <i>The 180-calendar-day verification time limit applies to files processed by the organization or its delegate(s) on or after July 1, 2025. Files processed before July 1, 2025, are scored against the previous verification time limit requirement of 365 calendar days.</i>		

Each file contains the application and attestation, and evidence of review by the organization's staff. Refer to CR 1, Element A, factor 13.

Factor 1: Inability to perform essential functions

The inquiry regarding inability to perform essential functions may vary or may exceed NCQA standards, depending on the organization's interpretation of applicable legal requirements such as the Americans with Disabilities Act (ADA).

Factor 2: Illegal drug use

Practitioners may use language other than "drug" to attest they do not use illegal substances. The organization may use more general or extensive language to query practitioners about impairment; language is not required to refer exclusively to the present or only to illegal substances.

Note: *Practitioners attest to their lack of present illegal drug use; past is not required.*

Factor 3:

History of loss of license

At initial credentialing, practitioners attest to any loss of license since their initial licensure. At recredentialing, practitioners attest to any loss of licensure since the last credentialing cycle.

History of felony convictions

At initial credentialing, practitioners attest to any felony convictions since their initial licensure. At recredentialing, practitioners attest to any felony convictions since the last credentialing cycle.

Factor 4: Limitation of privileges or disciplinary actions

At initial credentialing, practitioners attest to any loss or limitation of privileges or disciplinary actions since their initial licensure. At recredentialing, practitioners attest to any loss or limitation of privileges or disciplinary actions since the last credentialing cycle.

Factor 5: Current malpractice coverage

The application states the amount of a practitioner's current malpractice insurance coverage (even if the amount is \$0) and the date when coverage expires.

If the practitioner's malpractice insurance coverage is current and is provided in the application, it must be current as of the date when the practitioner signed the attestation and include the amount of coverage the practitioner has on the date when the attestation was signed. If the practitioner does not have current malpractice coverage, then it is acceptable to include future coverage with the effective and expiration dates.

Documentation of malpractice insurance coverage may also be a face sheet, a federal tort letter or employer professional liability policy as an addendum to the application. In this case, the practitioner is not required to attest to malpractice coverage on the application. The face sheet, federal tort letter or

employer professional liability policy must include the insurance effective and expiration dates (the future effective date is acceptable).

Evidence of a face sheet must:

- Be from the carrier, **and**
- Include the practice name, **and**
- Include a roster of all individuals in the practice who are covered under the policy. The roster may be provided from the carrier or employer.

Evidence of federal tort coverage must include effective and expiration dates but is not required to include a roster of all practitioners who are covered under the policy.

Evidence of private malpractice insurance coverage or employer professional liability policy must include a roster of all individuals in the practice who are covered under the policy.

Factor 6: Race, ethnicity and language

The organization's application includes fields to enter race, ethnicity and language, and a statement that the organization does not discriminate or base credentialing decisions on an applicant's race, ethnicity and language, and that providing the information is optional.

NCQA does not penalize the organization if the practitioner does not provide the information in the application.

Factor 7: Correctness and completeness of the application

If the application and attestation must be updated, only the practitioner may attest to the update; organization staff may not. If a copy of an application from an entity external to the organization is used, it must include an attestation to the correctness and completeness of the application. NCQA does not count the associated attestation elements as present if the practitioner did not sign the application within the required time frame.

Faxed, digital, electronic, scanned or photocopied signatures are acceptable. Signature stamps are not acceptable unless the practitioner is physically impaired and the disability is documented in the practitioner's file.

CAQH. CAQH has a system that allows the practitioner to update application information electronically. NCQA accepts the last attestation date generated by this system as the date when the practitioner signed and dated the application to attest to its completeness and correctness.

Note: CAQH requires practitioners to update the application every 120 calendar days. This meets the NCQA required time frame.

Exceptions

This element is NA for Interim Surveys.

For factor 6, applications processed before July 1, 2025 will be scored NA.

Related information

Meeting time limits. NCQA does not require receipt of the attestation before the organization conducts credentialing verification and queries required for other

elements. If the signature attestation exceeds the time limit before the credentialing decision, the practitioner must attest that the information on the application remains correct and complete, but is not required to complete another application. NCQA recommends that the organization send a copy of the completed application with the new attestation form when it requests the practitioner to update the attestation.

Use of other applications. The organization may use a state application or an application from another entity if it meets the factors in this element.

If state regulations require the organization to use a credentialing application that does not contain an attestation, or all information in factors 1–6, the organization attaches the attestation or additional information as an addendum to the application. If state regulations prohibit addenda to the application, the organization attaches a copy of the relevant regulations when it submits the survey tool.

Examples

Factor 1: Inability to perform essential functions questions

- Sufficient question to inquire about practitioner's ability to perform essential functions:
 - Are you currently suffering from any condition that impairs your judgment or that would otherwise adversely affect your ability to practice medicine in a competent, ethical and professional manner? (Yes/No).
- Insufficient question to inquire about practitioner's ability to perform essential functions:
 - Are you currently suffering from any condition (mental and/or physical) for which you are not being appropriately treated that impairs your judgement or that would otherwise adversely affect your ability to practice medicine in a competent, ethical, and professional manner?

Note: *This question is not acceptable because the question is limited to conditions a practitioner is not being appropriately treated for.*

Factor 2: Illegal drug use questions

- Sufficient question to inquire about practitioner's lack of present illegal drug use:
 - Are you currently using illegal drugs that could affect your ability to practice medicine?
- Insufficient question to inquire about practitioner's lack of present illegal drug use:
 - Are you currently using prescription drugs illegally?

Note: *This question is not acceptable because it is limited in scope. It does not cover illegal drugs.*

CR 4: Recredentialing Cycle Length

The organization formally recredentials its practitioners at least every 36 months.

Intent

The organization conducts timely recredentialing.

Element A: Recredentialing Cycle Length

The length of the recredentialing cycle is within the required 36-month time frame.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	High (90-100%) on file review	Medium (60-89%) on file review	Low (0-59%) on file review
Data source	Records or files		
Scope of review	Product lines <i>For Renewal Surveys: This element applies to all product lines.</i> Documentation NCQA reviews the timeliness of recredentialing within a random sample of up to 40 recredentialing files for practitioners that were due for recredentialing during the look-back period.		
Look-back period	<i>For Renewal Surveys: 36 months.</i>		
Explanation	THIS IS A MUST-PASS ELEMENT. This element applies to: • Practitioners in the scope of credentialing, as defined in <i>CR 1: Credentialing Policies</i>, Element A. • Recredentialing files, unless an exception noted below applies. Each file contains the Credentialing Committee decision date. The 36-month recredentialing cycle begins on the date of the previous credentialing decision. NCQA counts the 36-month cycle to the month, not to the day. Dispute of file review results NCQA conducts onsite file review in the presence of the organization's staff and works with the organization to resolve any disputes during the onsite survey. An organization that is unable to resolve a dispute with the survey team must contact NCQA before the onsite survey is complete. File review results may not be disputed or appealed once the onsite survey is complete.		

Exception

This element is NA for Interim and First Surveys.

Related information

Extending the recredentialing cycle length. The organization may extend a practitioner's recredentialing cycle time frame (beyond 36 months) if the practitioner is:

- On active military assignment.
- On medical leave (e.g., maternity leave).
- On sabbatical.

The organization documents this and recredentials the practitioner within 60 calendar days of the practitioner's return to practice.

Termination and reinstatement. If the organization terminates a practitioner for administrative reasons (e.g., the practitioner failed to provide complete credentialing information) and not for quality reasons, it may reinstate the practitioner within 30 calendar days of termination, and is not required to perform initial credentialing. The organization must perform initial credentialing if reinstatement is more than 30 calendar days after termination.

If the organization does not have the necessary information for recredentialing, it informs the practitioner that the information is needed at least 30 calendar days before the recredentialing deadline, and that without it, the practitioner will be administratively terminated. The organization includes this notification in the practitioner's credentialing file. If the practitioner is subsequently terminated for lack of information, the termination notice should be in the practitioner's file.

Failure to recredential within 36 months. The organization will be scored down if it missed the 36-month time frame for recredentialing a practitioner, but did not terminate the practitioner. The organization may recredential the practitioner within 30 calendar days of missing the deadline, but if recredentialing is not completed within 30 calendar days, the organization must initial credential the practitioner.

Termination of delegate. NCQA requires an unbroken string of recredentialing at least every 3 years. If an organization can obtain files from the delegate, it is not required to start over with initial credentialing; it may continue the process begun by the delegate, and recredential practitioners when they are due.

If the organization cannot obtain files from the delegate, it must perform initial credentialing within 6 months of the delegate's termination date. The organization is responsible for ensuring that credentialing occurs according to NCQA standards.

Examples

None.

CR 5: Ongoing Monitoring and Interventions

The organization develops and implements policies and procedures for ongoing monitoring of practitioner sanctions, complaints and quality issues between recredentialing cycles and takes appropriate action against practitioners when it identifies occurrences of poor quality.

Intent

The organization identifies and, when appropriate, acts on important quality and safety issues in a timely manner during the interval between formal credentialing.

Element A: Ongoing Monitoring

The organization implements ongoing monitoring by:

1. Collecting and reviewing Medicare and Medicaid sanctions.
2. Collecting and reviewing Medicare and Medicaid exclusions.
3. Collecting and reviewing sanctions, limitations and expiration on licensure.
4. Collecting and reviewing complaints.
5. Collecting and reviewing information from identified adverse events.

Summary of Changes

Clarifications

- Revised the look-back period for factor 2 and the factor 3 expiration component to 12 months.
- Added “Automatic credit is given for factor components included in the scope of an NCQA-Accredited/Certified delegate’s survey” as the fourth paragraph of the explanation.

Scoring	Met	Partially Met	Not Met
	The organization meets 4-5 factors	The organization meets 2-3 factors	The organization meets 0-1 factors

Data source Documented process, Reports, Materials

Scope of review

Product lines

For First Surveys and Renewal Surveys: This element applies to all product lines.

Documentation

NCQA reviews the organization’s policies and procedures and reports that demonstrate the organization collected and reviewed applicable information.

For factors 1–3, if the organization uses a monitoring service, NCQA also reviews the organization’s evidence of the subscription with the service.

Look-back period	<i>For First Surveys:</i> 6 months. <i>For Renewal Surveys:</i> 24 months, 12 months for factor 2 and the expiration component of factor 3.
Explanation	This element applies to practitioners in the scope of credentialing, as defined in <i>CR 1: Credentialing Policies</i>, Element A. The organization conducts ongoing monitoring between recredentialing cycles. The organization verifies sanction and exclusion information (from factors 1-3) for all product lines. Automatic credit is given for components of a factor that were included in the scope of an NCQA-Accredited/Certified delegate's survey. Factor 1: Sources for Medicare/Medicaid sanctions The organization obtains Medicaid sanction information from the State Medicaid agency and from one of the following additional sources: • AMA Physician Master File. • FSMB. • NPDB. • <u>SAM.gov</u>. The organization obtains Medicare sanction information from any of the following sources: • AMA Physician Master File. • FSMB. • NPDB. • <u>SAM.gov</u>. Factor 2: Sources for Medicare/Medicaid exclusions The organization obtains Medicaid exclusion information from the State Medicaid agency and from one of the following additional sources: • List of Excluded Individuals and Entities, maintained by OIG and available over the internet, or • NPDB. The organization obtains Medicare exclusion information from any of the following sources: • Medicare Exclusion Database. • List of Excluded Individuals and Entities maintained by OIG and available over the internet. • NPDB. Factor 3: Sources for licensure sanction, limitations and expiration information For all products, the organization collects and reviews sanction and limitations on licensure from at least one of the following sources, as appropriate:

- For all practitioner types:
 - NPDB.
 - Federal Employees Health Benefits Plan (FEHB) Program department record, published by the Office of Personnel Management, Office of the Inspector General.
- Physicians:
 - Appropriate state agencies.
 - FSMB.
- Chiropractors:
 - State Board of Chiropractic Examiners.
 - Federation of Chiropractic Licensing Boards' Chiropractic Information Network-Board Action Databank (CIN-BAD).
- Oral surgeons:
 - State Board of Dental Examiners or State Medical Board, depending on the state.
- Podiatrists:
 - State Board of Podiatric Examiners.
 - Federation of Podiatric Medical Boards.
- Nonphysician healthcare practitioners:
 - Appropriate state agency.
 - State licensure or certification board.

The organization collects and reviews expiration of licensure from the state licensing or certification agency (or its website).

Time frame for reviewing sanction, exclusions, limitations and expiration information

The organization reviews information from NCQA-approved sources listed above:

- At least every 30 calendar days, **or**
- Within 30 calendar days of a new alert, if the organization subscribes to a monitoring service (e.g., NPDB).

Expiration of licensure. The organization documents the license expiration date in the credentialing files or system, and monitors all licenses that will expire within the look-back period.

Factor 4: Collecting and reviewing complaints

The organization:

- Investigates all practitioner-specific member complaints upon their receipt, and evaluates the practitioner's history of complaints, if applicable, **and**
- Evaluates the history of all complaints for all practitioners at least every 6 months.

Factor 5: Adverse events

The organization monitors for adverse events:

- *Before July 1, 2025:* At least every 6 months.
- *On or after July 1, 2025:* At least monthly.

Adverse events include injury or harm while a member is receiving care from a practitioner.

Exception

This element is NA for Interim Surveys.

Examples**Factor 5: Adverse event**

A practitioner prescribes a medication for a member with a documented history of allergy to the medication, which leads to the member's hospitalization.

Element B: Appropriate Interventions

The organization reports the findings from Element A to the Credentialing Committee, or other designated peer-review body, and implements interventions as needed.

Summary of Changes**Clarifications**

- Revised the look-back period to 12 months for the "reporting of findings" component.
- Revised the scope of review to clarify NCQA review of meeting minutes.
- Revised the scope of review to clarify that if no occurrences were identified, NCQA reviews policies and procedures.

Scoring

Met	Partially Met	Not Met
The organization meets the requirement	No scoring option	The organization does not meet the requirement

Data source

Documented process, Reports

Scope of review**Product lines**

For First Surveys and Renewal Surveys: This element applies to all product lines.

Documentation

NCQA reviews the organization's policies and procedures for implementing appropriate interventions based on the information found in Element A.

If there were findings from Element A which the organization needs to address, NCQA also reviews credentialing committee or other designated peer-review body meeting minutes for evidence that findings were communicated and the committee or designated peer-review body recommended appropriate interventions, if applicable.

NCQA reviews up to three sets of credentialing committee or other designated peer-review body meeting minutes within the look-back period. If three sets of meeting minutes are not available, NCQA reviews all meeting minutes that are available from within the look-back period.

NCQA also reviews reports demonstrating the organization took action to address quality and safety issues throughout the look-back period, if applicable.

If no occurrences were identified, NCQA will only review policies and procedures for implementing appropriate interventions based on the information found in Element A.

Look-back period

For First Surveys: 6 months.

For Renewal Surveys: 24 months, 12 months for the “reporting of findings” component.

Explanation

This element applies to practitioners in the scope of credentialing, as defined in *CR 1: Credentialing Policies*, Element A.

The organization reports the findings to its credentialing committee or other designated peer-review body at the next meeting after the identified occurrence.

The credentialing committee or other designated peer-review body recommends appropriate interventions.

After determining which interventions are appropriate, the organization acts to address quality and safety issues.

The organization’s report includes the:

- Affected practitioner.
- Incident date.
- Quality issue.
- Date reported to the Credentialing Committee or other designated peer-review body.
- Organization’s actions.

Exception

This element is NA for Interim Surveys.

Examples

None.

CR 6: Notification to Authorities and Practitioner Appeal Rights

An organization that has taken action against a practitioner, for quality reasons, reports the action to the appropriate authorities and offers the practitioner a formal appeal process.

Intent

The organization uses objective evidence and patient-care considerations when deciding on a course of action for dealing with a practitioner who does not meet its quality standards.

Element A: Actions Against Practitioners

The organization has policies and procedures for:

1. The range of actions available to the organization.
2. Making the appeal process known to practitioners.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 2 factors	The organization meets 1 factor	The organization meets 0 factors
Data source	Documented process		
Scope of review	Product lines <i>For Interim Surveys and First Surveys: This element applies to all product lines.</i> Documentation NCQA reviews the organization's policies and procedures.		
Look-back period	<i>For Interim Surveys: Prior to the survey date.</i> <i>For First Surveys: 6 months.</i>		
Explanation	This element is a structural requirement . The organization must present its own documentation. This element applies to practitioners in the scope of credentialing, as defined in <i>CR 1: Credentialing Policies</i> , Element A. Factor 1: Range of actions available Policies and procedures specify: • The organization's process for reviewing network participation status of practitioners whose conduct could adversely affect members' health or welfare.		

- The range of actions that may be taken to improve practitioner performance before termination.
- That the organization's process for reporting actions to the appropriate authorities.

Factor 2: Making the appeal process known

No additional explanation required.

Exception

This element is NA for Renewal Surveys.

Examples

None.

CR 7: Assessment of Organizational Providers

The organization has written policies and procedures for initial and ongoing assessment of providers with which it contracts.

Intent

The organization evaluates the quality of providers with which it contracts.

Element A: Review and Approval of Provider

The organization's policy for assessing a health care delivery provider specifies that before it contracts with a provider, and for at least every 36 months thereafter, it:

1. Confirms that the provider is in good standing with state and federal regulatory bodies.
2. Confirms that the provider has been reviewed and approved by an accrediting body.
3. Conducts an onsite quality assessment if the provider is not accredited.

Summary of Changes

Clarifications

- Replaced “durable medical equipment entities” with “durable medical equipment suppliers” in the first paragraph of the explanation.
- Added an exception to clarify that this element is NA for Renewal Surveys.

Scoring	Met	Partially Met	Not Met
	The organization meets 2-3 factors	The organization meets 1 factor	The organization meets 0 factors
Data source	Documented process		
Scope of review	Product lines <i>For Interim Surveys and First Surveys: This element applies to all product lines.</i> Documentation NCQA reviews the organization's policies and procedures in place throughout the look-back period.		
Look-back period	<i>For Interim Surveys: Prior to the survey date.</i> <i>For First Surveys: 6 months.</i>		
Explanation	An organizational provider is a facility that provides services to members, and where members are directed for services rather than to a specific practitioner. This element applies to all organizational providers with which the organization contracts (e.g., telemedicine providers, urgent care centers, durable medical equipment suppliers). Facilities that do not provide services directly to		

members (e.g., durable medical equipment organizations that provide supplies to a practitioner's office) are not in the scope of review.

Factor 1: Confirmation with state and federal regulatory bodies

The organization's policies and procedures specify sources used to confirm that providers are in good standing with state and federal requirements, including:

- Applicable state or federal agency.
- Agent of the applicable state or federal agency.
- Copies of credentials (e.g., state licensure) from the provider.

NCQA does not accept an attestation from a provider to the organization about the provider's regulatory status.

Factor 2: Confirmation of review and approval by an accrediting body

The organization's policies and procedures specify sources used to confirm the provider's accreditation status, including:

- Applicable accrediting body for each type of organizational provider.
- Agent of the applicable accrediting body.
- Copies of credentials (e.g., accreditation report, certificate or decision letter) from the provider.

NCQA does not accept an attestation from a provider to the organization about the provider's accreditation status.

Factor 3: Site visits for unaccredited facilities

The organization's policies and procedures include:

- Onsite quality assessment criteria for each type of provider.
- A process ensuring that the providers credential their practitioners.

The organization receives credit for this factor if its policies and procedures specify that it contracts only with accredited providers.

If a provider has satellite facilities that follow the same policies and procedures as the provider, the organization may limit site visits to a main facility.

State or federal review in lieu of a site visit. The organization may have a policy to substitute a CMS or state quality review in lieu of a site visit under the following circumstances:

- The CMS or state review is no more than 3 years old.
 - If the CMS or state review is older than 3 years, the organization conducts its own onsite quality review.
- The organization obtains a survey report or letter from CMS or the state, from either the provider or the agency, stating that the facility was reviewed and passed inspection.
 - The report meets the organization's quality assessment criteria or standards.

The organization is not required to conduct a site visit if the provider is in a rural area, as defined by the U.S. Census Bureau (<https://www.hrsa.gov/rural-health/about-us/definition/datafiles.html>), and the state or CMS has not conducted a site review.

Exception

This element is NA for Renewal Surveys.

Related information

Time frame. NCQA does not prescribe a time frame for gathering data to use for assessing organizational providers (e.g., the 120-calendar-day rule, applied against the verification of credentials of individual practitioners, is NA).

Examples None.

Element B: Medical Providers

The organization includes at least the following medical providers in its assessment:

1. Hospitals.*
2. Home health agencies.
3. Skilled nursing facilities.
4. Free-standing surgical centers.

***Critical factors: Score cannot exceed Partially Met if one critical factor is scored “No.”**

Summary of Changes

Clarifications

- Added an exception to clarify that this element is NA for Renewal Surveys.

Scoring	Met	Partially Met	Not Met
	The organization meets 3-4 factors	The organization meets 2 factors	The organization meets 0-1 factors

Data source Documented process

Scope of review **Product lines**
For Interim Surveys and First Surveys: This element applies to all product lines.

Documentation

NCQA reviews an organization’s policies and procedures in place throughout the look-back period.

Look-back period *For Interim Surveys:* Prior to the survey date.
For First Surveys: 6 months.

Explanation Factor 1 is a critical factor; if it is scored “No,” the element score cannot exceed Partially Met.

Factors 2–4

No additional explanation required.

Exceptions

Factors 1–4 are NA if the organization does not contract with any provider types in factors 1–4.

This element is NA for Renewal Surveys.

Related information

Nonskilled home health agencies. Home health agencies that only provide home aides (e.g., for help with cooking, dressing, medical appointments) are not within the scope of CR 7.

Examples None.

Element C: Behavioral Healthcare Providers

The organization includes behavioral health care facilities providing mental health or substance abuse services in the following settings:

1. Inpatient.
2. Residential.
3. Ambulatory.

Summary of Changes

Clarifications

- Added an exception to clarify that this element is NA for Renewal Surveys.

Scoring	Met	Partially Met	Not Met
	The organization meets 3 factors	The organization meets 1-2 factors	The organization meets 0 factors

Data source Documented process

Scope of review **Product lines**

For Interim Surveys and First Surveys: This element applies to all product lines.

Documentation

NCQA reviews the organization’s policies and procedures in place throughout the look-back period.

Look-back period	<i>For Interim Surveys:</i> Prior to the survey date. <i>For First Surveys:</i> 6 months.
Explanation	Assessment policies and procedures address all applicable types of providers, regardless of how many members are treated at the facilities. The organization is not required to credential organizational providers that operate only as 12-step programs. Exceptions This element is NA: • If all purchasers of the organization's services carve out or exclude behavioral healthcare. • For Renewal Surveys. Factors 1–3 are NA if the organization does not contract with any provider types in factors 1–3. Factor 2 is NA if residential treatment facilities are not part of the organization's benefit package or are unavailable in the service area.
Examples	Behavioral healthcare providers • Psychiatric hospitals and clinics. • Addiction disorder facilities. • Residential treatment centers for psychiatric and addiction disorders.

Element D: Assessing Medical Providers

The organization assesses contracted medical health care providers against the requirements and within the time frame in Element A.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement

Data source	Reports, Records or files
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Scope of review	Product lines
	<i>For First Surveys and Renewal Surveys: This element applies to all product lines.</i>
	Documentation

NCQA reviews evidence that the organization assessed the providers in Element B. The organization provides documentation of a tracking mechanism(s) (checklist or spreadsheet); a separate tracking mechanism or report is not required for each provider.
--

Look-back period *For First Surveys:* 6 months.
For Renewal Surveys: 24 months.

Explanation The organization is not required to conduct a site visit if the provider is in a rural area, as defined by the U.S. Census Bureau (<https://www.hrsa.gov/rural-health/about-us/definition/datafiles.html>), and the state or CMS has not conducted a site review.

Exception

This element is NA for Interim Surveys.

Examples

Table 1: Assessment of organizational providers tracking log

Organization Name	Organization Type	Confirmation Dates and Statuses		
		Licensing & Regulatory	Accrediting Body	Site Visit
Mega	Hospital	4/1/2023; Active	4/10/2023; Name; Active	Rural provider, visit not conducted by state or CMS.
		4/5/2026; Active	4/15/2026; Name; Active	Rural provider, visit not conducted by state or CMS.
Downtown Surgery Center	Free-Standing Surgical Center	3/2/2023 Active	None	2/10/2023; CMS Compliant
		3/15/2026; Active	None	2/2/2026; CMS Compliant
District Physicians	Home Health	3/2/2023; Active	3/20/2023; Name; Active	Rural provider, visit not conducted by state or CMS.
		3/17/2026; Active	3/20/2026; Name; Active	Rural provider, visit not conducted by the state or CMS.

Element E: Assessing Behavioral Healthcare Providers

The organization assesses contracted behavioral healthcare providers against the requirements and within the time frame in Element A.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement

Data source Reports, Records or files

Scope of review **Product lines**
For First Surveys and Renewal Surveys: This element applies to all product lines.

Documentation

NCQA reviews evidence that the organization assessed the providers in Element C. The organization provides documentation of a tracking mechanism(s) (checklist or spreadsheet); a separate tracking mechanism or report is not required for each provider.

Look-back period *For First Surveys: 6 months.*
For Renewal Surveys: 24 months.

Explanation The organization is not required to conduct a site visit if the provider is in a rural area, as defined by the U.S. Census Bureau (<https://www.hrsa.gov/rural-health/about-us/definition/datafiles.html>), and the state or CMS has not conducted a site review.

The organization is also not required to conduct a site visit of ambulatory facilities that are not part of the organization's benefits package or are not available in the service area.

Exceptions

This element is NA:

- If all purchasers of the organization's services carve out or exclude behavioral healthcare.
- For Interim Surveys.

The organization provides evidence to support the score of "NA."

Examples**Table 1: Assessment of behavioral healthcare organizational providers tracking log**

Organization Name	Organization Type	Confirmation Dates and Statuses		
		Licensing & Regulatory	Accrediting Body	Site Visit
Mega X	Ambulatory	4/1/2023; Active	4/10/2023; Name; Active	NA
		4/5/2026; Active	4/15/2026; Name; Active	NA
Getting Better	Residential	3/2/2023; Active	None	2/2/2023; CMS Compliant
		3/15/2026; Active	None	2/10/2026; CMS Compliant

CR 8: Credentialing Information Integrity

The organization has credentialing information integrity policies and procedures, audits credentialing information for inappropriate documentation and updates and implements corrective actions that address identified information integrity issues.

Intent

The organization demonstrates its commitment to protecting the integrity of credentialing information used in the credentialing process.

Element A: Protecting the Integrity of Credentialing Information

The organization has credentialing information integrity policies and procedures that specify:

1. The scope of credentialing information.
2. The staff responsible for performing credentialing activities.
3. The process for documenting updates to credentialing information.
4. Inappropriate documentation and updates.
5. The organization audits CR staff and the process for documenting and reporting identified information integrity issues.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 4-5 factors	No scoring option	The organization meets 0-3 factors
Data source	Documented process		
Scope of review	Product lines <i>This element applies to Interim Surveys, First Surveys and Renewal Surveys for all product lines.</i> Documentation NCQA reviews the organization's policies and procedures for protecting the integrity of credentialing information.		
Look-back period	For All Surveys: Prior to the survey date.		
Explanation	THIS IS A MUST-PASS ELEMENT. This element is a structural requirement . The organization must present its own documentation. This element applies to credentialing information (both paper and electronic) used in the credentialing process.		

Credentialing information integrity refers to maintaining and safeguarding the information used in the initial credentialing and recredentialing process against inappropriate documentation and updates.

The organization's credentialing information integrity policies and procedures specifically address the integrity of information used in the credentialing process.

Factor 1: Scope of credentialing information

The organization's policies and procedures specify protection of each of the following types of credentialing information:

- The practitioner application and attestation.
- Credentialing documents received from the source or agent.
- Documentation of credentialing activities:
 - Verification dates.
 - Report dates (e.g., sanctions, complaints, identified adverse events).
 - Credentialing decisions.
 - Credentialing decision dates.
 - Signature or initials of the verifier or reviewer.
- Credentialing Committee minutes.
- Documentation of clean file approval, if applicable.
- Credentialing checklist, if used.

Factor 2: Staff responsible for performing credentialing activities

The organization's policies and procedures specify the titles of staff who are:

- Responsible for documenting credentialing activities.
- Authorized to modify (edit, update, delete) credentialing information.
 - Policies and procedures state if no staff are authorized to modify credentialing information under any circumstances.
- Responsible for oversight of credentialing information integrity functions, including auditing.

Factor 3: Process for documenting updates to credentialing information

The organization's policies and procedures:

- Specify when updates to existing credentialing information is appropriate (e.g., to update expiring credentials).
- Describe the organization's process for documenting the following when updates are made to credentialing information:
 - When (date and time) the information was updated.
 - What information was updated.
 - Why the information was updated.
 - Staff who updated the information.

Factor 4: Inappropriate documentation and updates

The organization's policies and procedures specify that the following documentation and updates to credentialing information are inappropriate:

- Falsifying credentialing dates (e.g., licensure date, credentialing decision date, staff verifier date, ongoing monitoring dates).
- Creating documents without performing the required activities (e.g., photocopying a prior credential and updating information as a new credential).
- Fraudulently altering existing documents (e.g., credentialing minutes, clean-file reports, ongoing monitoring reports).
- Attributing verification or review to an individual who did not perform the activity.
- Updates to information by unauthorized individuals.

Factor 5: Auditing, documenting and reporting information integrity issues

The organization's policies and procedures:

- Specify that the organization audits credentialing staff documentation and updates.
 - The organization does not have to include the audit methodology, but must indicate that an annual audit is performed.
- Describe the process for documenting and reporting inappropriate documentation and updates to:
 - The organization's designated individual(s) when identified, **and**
 - NCQA, when the organization identifies fraud and misconduct.
 - Self-identification of systemic issues affecting 5% or more of eligible credentialing/recredentialing files; for example, falsifying of verification dates. Refer to Section 5: Notifying NCQA of Reportable Events in the Policies and Procedures for details.
- Specify consequences for inappropriate documentation and updates.

Exceptions

None.

Examples

None.

Element B: Information Integrity Training

The organization annually trains credentialing staff on:

1. Inappropriate documentation and updates (Element A, factor 4).
2. Organization audits of staff, documenting and reporting information integrity issues (Element A, factor 5).

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 2 factors	No scoring option	The organization meets 0-1 factors
Data source	Reports, Materials		
Scope of review	Product lines <i>This element applies to Interim Surveys, First Surveys and Renewal Surveys for all product lines.</i> Documentation <i>For Interim Surveys:</i> NCQA reviews training materials. <i>For First and Renewal Surveys:</i> NCQA reviews training materials and evidence that the organization conducted the required training.		
Look-back period	<i>For All Surveys:</i> At least once during the prior year.		
Explanation	This element is a structural requirement. The organization must present its own documentation. Factor 1: Inappropriate documentation and updates The organization trains credentialing staff on inappropriate documentation and updates to credentialing information, as defined in Element A, factor 4. Factor 2: Auditing, documenting and reporting information integrity issues The organization's training informs credentialing staff of: • Organization audits of staff documentation and updates in credentialing files. • The process for documenting and reporting inappropriate documentation and updates to: – The organization's designated individual(s) when identified. – NCQA, when the organization identifies fraud and misconduct, as identified in Element A, factor 4. • The consequences for inappropriate documentation and updates.		

Exceptions

None.

Examples

None.

Element C: Audit and Analysis

The organization annually:

1. Audits for inappropriate documentation and updates to credentialing information.
2. Conducts qualitative analysis of inappropriate documentation and updates.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 2 factors	No scoring option	The organization meets 0-1 factors

Data source Reports

Scope of review **Product lines**

This element applies to First Surveys and Renewal Surveys for all product lines.

Documentation

For First Surveys and Renewal Surveys: NCQA reviews the organization's audit and analysis reports completed during the look-back period.

Look-back period *For First Surveys and Renewal Surveys:* At least once during the prior year.

Explanation This element is a **structural requirement**. The organization must present its own documentation.

Factor 1: Audit

The organization annually audits credentialing information used in the credentialing process for the following inappropriate documentation and updates:

- Falsifying credentialing dates (e.g., licensure dates, credentialing decision dates, staff verifier dates, ongoing monitoring dates).
- Creating documents without performing the required activities.
- Fraudulently altering existing documents (e.g., credentialing minutes, clean-file reports, ongoing monitoring reports).
- Attributing verification or review to an individual who did not perform the activity.
- Updates to information by unauthorized individuals.

The audit universe includes practitioner files for all initial credentialing decisions and all recredentialing decisions made or due during the look-back period. The organization randomly audits a sample of practitioner files from the audit universe using 5% or 50 files, whichever is less.

The random sample includes at least 10 credentialing files and 10 recredentialing files. If fewer than 10 practitioners were credentialed or recredentialled within the look-back period, the organization audits all files. The organization may choose to audit more practitioner files than NCQA requires.

The organization provides an auditing and analysis report that includes:

- The report date.
- The title of individuals who conducted the audit.
- The 5% or 50 files auditing methodology.
 - Auditing period.
 - File audit universe size (described in the paragraph above).
 - Audit sample size.
- The audit log (as a referenced attachment).
 - File identifier (individual practitioner).
 - Type of credentialing information audited (e.g., licensure).
- Findings for each file.
 - A rationale for inappropriate documentation and updates.
- The number or percentage and total inappropriate documentation and updates by type of credentialing information.

The organization must provide a completed audit report even if no inappropriate documentation and updates were found.

Factor 2: Qualitative analysis

The organization annually conducts qualitative analysis of each instance of inappropriate documentation and update identified in the audit (factor 1) to determine the cause.

The organization's auditing and analysis report also includes:

- Titles of credentialing staff involved in the qualitative analysis.
- The cause of each finding.

Refer to *Appendix 4: Glossary* for the full definition of qualitative analysis.

Exceptions

This element is NA for Interim Surveys.

Factor 2 is NA if the organization did not identify any inappropriate documentation and updates (factor 1). NCQA assesses whether this conclusion is reasonable, based on results of the organization's analysis.

Related information

If the organization audits more frequently, it must use the "5% or 50 files" methodology for each audit, and all audits must cumulatively cover the 12-month look-back period.

Examples Factors 1, 2: Audit and analysis report

[Organization's name]—Annual CR Information Integrity Assessment Report

The report date: February 10, [current year].

Auditor: [Individual's Name], Credentialing Director

Auditing methodology: Each January, the credentialing director audits a random sample of initial and recredentialing files for inappropriate documentation and updates for the previous calendar year. The organization randomly samples and audits 5% or 50 files (whichever is less) of all credentialing decisions made or due in the previous year.

- Period reviewed: January 1, [previous year]–December 31 [previous year].
- File-review universe size: The organization initial credentialed 2,000 practitioners, and recredentialed 8,000 practitioners who were due for recredentialing in the previous year.
- Audit sample size calculation: 10,000 practitioner files x 0.05 = 500 files.
- Audit sample size: 50 files, which is less than the 5% (500 files). The organization audited a random sample of 50 files that included 20 initial credentialing files and 30 recredentialing files.

Audit date: January 6–9, [current year].

Audit log: Attachment I. Partial illustration in table below.

Practitioner ID	File Type (Initial/Recred)	Inappropriate Documentation/ Updates	Credential Affected	Finding
Practitioner 1	Recredential	No		NA
Practitioner 2	Initial	No		NA
Practitioner 3	Recredential	Yes	Attestation	Attestation date updated by staff (name) instead of practitioner because attestation was expiring. 3/4/[previous year] @ 2:59 PM
Practitioner 4	Recredential	Yes	Licensure Sanction Information	Verification of licensure and sanction information updated by staff (name) without going to the source (3/4/[previous year] @ 11:00 AM) because the committee meeting was scheduled for the next day.

Summary of findings:

Credentialing Information Reviewed	Noncompliant Initial Credentialing Files	Noncompliant Recredentialing Files	Percentage of Noncompliant Modifications
Application and Attestation	4	4	16%
License	2	2	8%
DEA/CDS	0	0	0%
Education and Training	0	NA	0%
Board Certification Status	0	0	0%
Work History	0	NA	0%
Malpractice History	0	0	0%
Sanction Information	2	2	8%
Credentialing Committee Minutes	0	0	0%
Clean-File Approvals	0	0	0%
Ongoing Monitoring Reports	0	0	0%

Qualitative analysis. The credentialing director provided the credentialing staff with the audit log documenting how, when, and by whom files were updated.

The credentialing director held a series of meetings (January 14–17, [current year]) with credentialing staff (credentialing assistant director, credentialing manager, credentialing analyst) to determine the causes of each inappropriate update. The causes of the inappropriate updates are outlined in the table below.

Credentialing Information	Description of Noncompliant Update	Reason
Application and Attestation	Attestation date updated by staff instead of practitioner.	Staff spoke with the practitioner, who stated that all information remained accurate. Staff did not know that only the practitioner can update the information.
License	Verification was updated without going to the source.	Staff responsible for verification of licensure and sanction information was on emergency leave and did not complete the verification. Because temporary staff did not have time to complete verification of all practitioners, they copied existing credentials, changed dates and uploaded the information into the CR system before the Credentialing Committee meeting.

Element D: Improvement Actions

The organization:

1. Implements corrective actions to address all inappropriate documentation and updates found in Element C.
2. Conducts an audit of the effectiveness of corrective actions (factor 1) on findings 3–6 months after completion of the annual audit in Element C.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 2 factors	No scoring option	The organization meets 0-1 factors
Data source	Documented process, Reports, Materials		
Scope of review	Product lines <i>This element applies to First Surveys and Renewal Surveys for all product lines.</i>		
	Documentation <i>For First Surveys and Renewal Surveys:</i> • <i>For factor 1:</i> NCQA reviews the organization's documentation of corrective actions planned or taken to address inappropriate documentation and updates. • <i>For factor 2:</i> NCQA reviews the organization's audit of the effectiveness of corrective actions.		
Look-back period	<i>For First Surveys and Renewal Surveys:</i> At least once during the prior year.		
Explanation	This element is a structural requirement. The organization must present its own documentation. The organization addresses credentialing information integrity issues identified in Element C. Factor 1: Implement corrective actions The organization documents corrective actions taken or planned, including dates of actions, to address all inappropriate documentation and updates (findings) identified in Element C. One action may address more than one finding, if appropriate. Annual training (Element B) may not be the only corrective action. The organization identifies staff (by title) who are responsible for implementing corrective actions.		

Factor 2: Measure of effectiveness follow-up audit

The organization audits the effectiveness of corrective actions (factor 1) on findings within 3–6 months of the annual audit completed for Element C and draws conclusions about the actions' overall effectiveness. The audit universe includes practitioner files for all credentialing decisions made, or due to be made, 3–6 months after the annual audit.

The organization conducts a qualitative analysis if it identifies noncompliance with integrity policies and procedures during the follow-up audit.

Exceptions

This element is NA for Interim Surveys.

This element is NA if the organization did not identify any inappropriate documentation and updates, according to the audit and analysis report reviewed for Element C. NCQA assesses whether this conclusion is reasonable, based on results in the organization's audit and analysis report.

Factor 2 is NA if the annual audit is less than 3 months before the organization's NCQA Survey.

Examples**Factor 1: Implement Corrective Actions**

[Organization Name]'s Credentialing Director shared the audit analysis results and mitigation recommendations with the organization's leadership on January 31, [current year]. [Organization Name] leadership required immediate implementation of actions and completion of all corrective actions, outlined in the table below, on or before the dates specified.

Credentialing Information/ Noncompliant Update	Reason	Correction Actions Planned
Application and Attestation: Attestation date updated by staff instead of by practitioner.	Staff spoke with the practitioner, who stated that all information remained accurate. Staff did not know that only the practitioner can update the information.	Educate credentialing staff on the organization's policies and procedures by 4/15/[current year]. Owner: Credentialing Director. Train credentialing staff on NCQA's documentation requirements by 4/15/[current year]. Owner: Credentialing Director. Establish automated resending of attestation to practitioner 60 days before expiration by 4/15/[current year]. Owner: Credentialing Director.

Credentialing Information/ Noncompliant Update	Reason	Correction Actions Planned
License: Verification was not updated from the source.	Staff responsible for verification of licensure and sanction information was on emergency leave and did not complete verification.	Require credentialing staff to undergo ethics training, with emphasis on following the organization's processes even if under pressure to take shortcuts by 5/1/[current year]. Owner: Credentialing Director.
Sanction Information: Verification was not updated from the source.	Because temporary staff did not have time to complete verification of all practitioners, they copied existing credentials, changed dates and uploaded the information into the CR system before the Credentialing Committee meeting.	Incorporate system flags that do not allow updating information without going to the source and require confirmation that the information was received from the source by 6/1/[current year]. Owner: IT Director, Credentialing Director. Purchase software application to automatically retrieve verification from accepted sources (web crawler) by 6/1/[current year]. Owner: IT Director, Credentialing Director.

Factor 2: Effectiveness of corrective actions report

[Organization name]—Annual CR Information Integrity Measure of Effectiveness

Report date: August 30 [current year].

Auditor: [Individual's name], Credentialing Director

Auditing methodology: [Organization name] audits the effectiveness of corrective actions taken 6 months of the annual audit completed January 6–9, [current year], by randomly selecting a sample of 5% or 50 files (whichever is less) initial and recredentialing files for inappropriate documentation and updates since the last annual audit.

- Period reviewed: February [current year]–July [current year].
- File-review universe size: 5,000 files.
- Audit sample size calculation: 5,000 practitioner files x 0.05 = 250 files.
- Audit sample size: 50 files, which is less than the 5% (250 files). The organization reviewed a random sample of 20 initial credentialing files and 30 recredentialing files.

Audit date: July 11–15 [current year].

Audit log: Attachment I. (Not shown in this example.)

Summary of findings:

Credentialing Information Reviewed	Noncompliant Initial Credentialing Files	Noncompliant Recredentialing Files	Percentage of Noncompliant Modifications
Application and Attestation	0	0	0%
License	0	0	0%
Sanction Information	0	0	0%

Qualitative analysis: Not required.

Actions' effectiveness:

Noncompliant Credentialing Updates	Corrective Actions Completed	Action Effectiveness Audit
Application and Attestation: Attestation date updated by staff instead of by practitioner.	All staff completed education on the organization's policies and procedures on 4/15/[current year]. All staff completed training on NCQA documentation requirements on 4/15/[current year]. Established automated resending of attestation to practitioner 60 days before expiration on 4/15/[current year].	These actions have eliminated updating of attestation by staff. There were no incidences identified in the audit.
License: Verification was not updated from the source.	All credentialing staff completed ethics training, with emphasis on following the organization's processes even if under pressure to take shortcuts on 5/1/[current year]. Incorporated system flags that do not allow updating information without going to the source and require confirmation that the information was received from the source 6/1/[current year]. Purchased software application to automatically retrieve verification from accepted sources (web crawler) 6/1/[current year].	The were no incidences identified in the audit.
Sanction Information: Verification was not updated from the source.		

Overall effectiveness—Conclusion: The corrective actions implemented have been effective in preventing inappropriate documentation and updates based on follow-up assessment and the fact that no incidences of inappropriate documentation and updates were made.

CR 9: Delegation of CR

If the organization delegates any NCQA-required credentialing activities, there is evidence of oversight of the delegated activities.

Intent

The organization remains responsible for credentialing and recredentialing its practitioners, even if it delegates all or part of these activities.

Element A: Delegation Agreement

The written delegation agreement:

1. Is mutually agreed upon.
2. Describes the delegated activities and the responsibilities of the organization and the delegated entity.
3. Requires at least semiannual reporting by the delegated entity to the organization.
4. Describes the process by which the organization evaluates the delegated entity's performance.
5. Specifies that the organization retains the right to approve, suspend and terminate individual practitioners, providers and sites, even if the organization delegates decision making.
6. Describes the remedies available to the organization if the delegated entity does not fulfill its obligations, including revocation of the delegation agreement.

Summary of Changes

Clarifications

- Added a paragraph under the related information to refer to Appendix 2: Delegation and Automatic Credit Guidelines

Policy Changes

- Added a bullet under the "For factor 4" section in the scope of review to provide advance notice that all delegation agreements must address the UM Information Integrity requirements for surveys on or after July 1, 2027.

Scoring	Met	Partially Met	Not Met
	The organization meets 5-6 factors	The organization meets 3-4 factors	The organization meets 0-2 factors
Data source	Materials		
Scope of review	Product lines <i>For Interim Surveys, First Surveys and Renewal Surveys: This element applies to all product lines.</i>		

Documentation

NCQA reviews delegation agreements in effect during the look-back period from up to four randomly selected delegates, or reviews all delegates if the organization has fewer than four.

For factor 4:

- New delegation agreements implemented on or after July 1, 2025, must address the delegate's credentialing information integrity.
- Delegation agreements in place prior to July 1, 2025, that address the system controls under the 2022–2024 standards do not need to be updated to address credentialing information integrity requirements. The agreement must address systems controls, but NCQA does not evaluate the agreement against system controls requirements specified in prior years.
- Delegation agreements in place prior to July 1, 2025, that do not address the system controls intent under the 2022–2024 standards must be updated prior to July 1, 2025, to address credentialing information integrity requirements.
- All delegation agreements for surveys starting on or after July 1, 2027, must address the UM Information Integrity requirements.

The score for the element is the average of the scores for all delegates.

Look-back period

For Interim Surveys and First Surveys: 6 months; prior to the survey date for the information integrity component under factor 4.

For Renewal Surveys: 24 months; prior to the survey date for the information integrity component under factor 4.

Explanation

This element may not be delegated.

This element applies to agreements that are in effect within the look-back period.

The delegation agreement describes all delegated credentialing activities. A generic policy statement about the content of delegated arrangements does not meet this element.

Factor 1: Mutual agreement

Delegation activities are mutually agreed on before delegation begins, in a dated, binding document or communication between the organization and the delegated entity.

NCQA considers the effective date specified in the delegation agreement as the mutually agreed-upon effective date. The effective date may be before or after the signature date on the agreement. If the agreement has no effective date, NCQA considers the signature date (the date of the last signature) as the mutually agreed upon effective date.

NCQA may accept other evidence of the mutually agreed-upon effective date: a letter, meeting minutes or other form of communication between the organization and the delegate that references the parties' agreement on the effective date of delegated activities.

NCQA requires submitted evidence for all other delegation factors to consider the same mutually agreed-upon date as the effective date for the delegate's performance of delegated activities.

Factor 2: Assigning responsibilities

The delegation agreement or an addendum thereto or other binding communication between the organization and the delegate specifies credentialing activities:

- Performed by the delegate, in detailed language.
- Not delegated, but retained by the organization.
 - The organization may include a general statement in the agreement addressing retained functions (e.g., the organization retains all other credentialing functions not specified in this agreement as the delegate's responsibility).

If the delegate subdelegates an activity, the delegation agreement must specify which organization is responsible for oversight of the subdelegate.

Factor 3: Reporting

The organization determines the method of reporting and the content of the reports, but the agreement must specify:

- That reporting is semiannual.
- The information is reported the delegate about delegated activities.
- How, and to whom, information is reported (i.e., joint meetings, or to appropriate committees or individuals in the organization).

The organization must receive regular reports from all delegates, including NCQA-Accredited delegates. NCQA scores this factor "Yes" if the organization delegates credentialing activities to an NCQA-Certified CVO that is certified to perform the delegated activity.

Factor 4: Performance monitoring

The delegation agreement states the organization's process for monitoring and evaluating the delegate's performance, as required in Element C, including credentialing information integrity.

Credentialing information integrity refers to maintaining and safeguarding the information used in the initial credentialing and recredentialing process against inappropriate documentation and updates, as outlined in *CR 8: Credentialing Information Integrity*, Element A, factor 4.

If the organization delegates any credentialing functions or activities covered in the scope of these standards, the delegate protects the integrity of the credentialing information used in the credentialing process. The delegation agreement specifies that the following documentation and updates to credentialing information are inappropriate:

- Falsifying credentialing dates (e.g., licensure date, credentialing decision date, staff verifier date, ongoing monitoring dates).
- Creating documents without performing the required activities.

- Fraudulently altering existing documents (e.g., credentialing minutes, clean-file reports, ongoing monitoring reports).
- Attributing verification or review to an individual who did not perform the activity.
- Updates to information by unauthorized individuals.

Factor 5: Right to approve, suspend and terminate

No additional explanation required.

Factor 6: Consequences for failure to perform

The delegation agreement specifies consequences if a delegate fails to meet the terms of the agreement and, at a minimum, circumstances that result in revocation of the agreement.

Exception

This element is NA if the organization does not delegate credentialing activities.

Related information

Outsourcing credentialing data storage to a cloud-based entity. It is not considered delegation if the organization only outsources credentialing data storage to a cloud-based entity that does not provide services that create, modify or use the credentialing data.

Refer to *Appendix 2: Delegation and Automatic Credit Guidelines* for additional information.

Examples

Factor 3: Reporting for delegation of credentialing

- Lists of credentialed and recredentialed practitioners.
- Committee meeting minutes.
- List of providers assessed.

Element B: Predelegation Evaluation

For new delegation agreements initiated in the look-back period, the organization evaluated delegate capacity to meet NCQA requirements before delegation began.

Summary of Changes

Clarifications

- Added “Amended agreements” subhead and text in last paragraph under “Predelegation evaluation.”

Scoring

Met	Partially Met	Not Met
The organization evaluated delegate capacity before delegation began	The organization evaluated delegate capacity after delegation began	The organization did not evaluate delegate capacity

Data source	Reports
Scope of review	Product lines <i>For Interim Surveys, First Surveys and Renewal Surveys: This element applies to all product lines.</i> This element applies if delegation was implemented in the look-back period. Documentation NCQA reviews the organization's predelegation evaluation from up to four randomly selected delegates, or reviews all delegates if the organization has fewer than four. The score for the element is the average of the scores for all delegates.
Look-back period	<i>For Interim Surveys and First Surveys: 6 months.</i> <i>For Renewal Surveys: 12 months.</i>
Explanation	This element may not be delegated. NCQA-Accredited/Certified delegates For non-information integrity requirements, automatic credit is available for this element if all delegates are NCQA-Accredited health plans or MBHOs, or are NCQA Accredited in CR or NCQA-Certified in CR or are NCQA-Certified CVOs, unless the element is NA. Organizations that are NCQA-Certified in CR or are Certified CVOs must be certified to perform the activity delegated by the organization. For information integrity requirements (CR 8, Elements A–D), automatic credit is available for this element if all delegates are NCQA-Accredited health plans or MBHOs, or are NCQA Accredited in CR or are NCQA-Certified CVOs under the 2025 standards or beyond, unless the element is NA. Note: <i>For organizations that have both NCQA-Accredited/Certified and non-Accredited/Certified delegates:</i> • <i>NCQA-Accredited/Certified delegates are eligible for automatic credit.</i> • <i>Non-Accredited/Certified delegates are reviewed and scored accordingly.</i> Predelegation evaluation The organization evaluated the delegate's capacity to meet NCQA requirements within 12 months prior to implementing delegation. The evaluation may include a review of the delegate's structure, processes and staffing in order to determine its capability to perform the delegated function. NCQA considers the date of the agreement to be the implementation date if the delegation agreement does not include an implementation date. If the time between the predelegation evaluation and implementation of delegation exceeds 12 months, the organization conducts another predelegation evaluation.

If the organization amends the delegation agreement to include additional credentialing activities within the look-back period, it performs a predelegation evaluation for the additional activities.

Amended agreements

If the organization amends the delegation agreement to include additional CR activities within the look-back period, it performs a predelegation evaluation for the additional activities before the implementation date of the new or amended agreement.

Exceptions

This element is NA if:

- The organization does not delegate credentialing activities.
- Delegation arrangements have been in effect for longer than the look-back period.

Related information

Use of collaboratives. An organization may collaborate in a statewide predelegation evaluation with other organizations that have overlapping practitioner and provider networks. The organizations in the collaborative use the same audit tool and share data. Each organization is responsible for meeting NCQA delegation standards, but may use the shared data collection process to reduce burden.

Examples

Predelegation evaluation

- Site visit.
- Telephone consultation.
- Documentation review.
- Committee meetings.
- Virtual review.

Element C: Review of Delegate's Credentialing Activities

For delegation arrangements in effect for 12 months *or* longer, the organization:

1. Annually reviews its delegate's credentialing policies and procedures.
2. Annually audits credentialing and recredentialing files against NCQA standards for each year that delegation has been in effect.
3. Annually evaluates delegate performance against NCQA standards for delegated activities.
4. Semiannually evaluates regular reports, as specified in Element A.
5. Annually audits each delegate's credentialing files for inappropriate documentation and inappropriate updates to credentialing information.
6. Implements corrective actions to address inappropriate documentation and inappropriate updates found in factor 5, for each delegate.

7. Conducts an audit of the effectiveness of corrective actions (factor 6) on the findings for each delegate 3–6 months after completion of the annual audit for factor 5.

Summary of Changes

Clarifications

- Added a subhead and text regarding amended agreements in the second paragraph of the explanation.

Scoring

Met	Partially Met	Not Met
The organization meets 6-7 factors	The organization meets 4-5 factors	The organization meets 0-3 factors

Data source

Reports

Scope of review

Product lines

For Interim Surveys: Factor 1 applies to all product lines.

For First Surveys and Renewal Surveys: This element applies to all product lines.

Documentation

NCQA reviews evidence of the organization's review from up to four randomly selected delegates, or all delegates if the organization has fewer than four.

For All Surveys: NCQA reviews the organization's evaluation of the delegate's credentialing policies and procedures (factor 1).

For First Surveys: NCQA also reviews the organization's most recent semiannual evaluation, annual review, audits, performance evaluation, corrective actions and measure of effectiveness (factors 2–7).

For Renewal Surveys:

- Factors 2–4:* NCQA also reviews the organization's most recent and the previous year's annual reviews, audits, performance evaluations and four semiannual evaluations.
- Factors 5–7:* NCQA also reviews the organization's most recent annual audit, performance evaluation, corrective actions and measure of effectiveness.

The score for the element is the average of the scores for all delegates.

Look-back period

For Interim Surveys and First Surveys: At least once during the prior year.

For Renewal Surveys: 24 months for factors 1–4; at least once during the prior year for factors 5–7.

Explanation This element may not be delegated.

Amended agreements

If the organization amends the delegation agreement to include additional activities within 12 months prior to the survey date, it is not required to complete this element for the new activities. NCQA reviews the organization's predelegation evaluation of new delegation activities in Element B.

NCQA-Accredited/Certified delegates

Automatic credit is available for factors 2 and 3 if all delegates are NCQA-Accredited health plans or MBHOs, NCQA Accredited in CR, NCQA-Certified in CR or are NCQA-Certified CVOs, unless the element is NA.

Organizations that are NCQA-Certified in CR or are Certified CVOs must be certified to perform the activity delegated by the organization.

Automatic credit for factor 4 is available for organizations NCQA-Certified in CR or Certified CVOs that are certified to perform the delegated activity.

For factors 5–7, automatic credit is available if all delegates are NCQA-Accredited under the 2025 standards (or later) for Health Plan Accreditation, MBHO Accreditation, CR Accreditation or CR Certification, unless the element is NA.

Note: *For organizations that have both NCQA-Accredited/Certified and non-Accredited/Certified delegates:*

- *NCQA-Accredited/Certified delegates are eligible for automatic credit.*
- *Non-Accredited/Certified delegates are reviewed and scored accordingly.*

Factor 1: Review of credentialing policies and procedures

The appropriate organization staff or committee reviews the delegate's credentialing policies and procedures. At a minimum, the organization reviews the sections of the policies and procedures that apply to the delegated functions.

Factor 2: Annual file audit

If the organization delegates credentialing, it audits the delegate's credentialing and recredentialing files against NCQA standards.

Note: *The organization may use the same file sample for factors 2 and 5.*

The organization uses one of the following methods to audit the files:

- 5% or 50 of its files, whichever is less, to ensure that information is verified appropriately.
 - At a minimum, the sample includes at least 10 credentialing files and 10 recredentialing files. If fewer than 10 practitioners were credentialed or recredentialed since the last annual audit, the organization audits the universe of files rather than a sample.
- The NCQA "8/30 methodology" available at <https://www.ncqa.org/programs/health-plans/policy-accreditation-and-certification/>

The organization bases its annual audit on the responsibilities of the delegate described in the delegation agreement and the appropriate NCQA standards.

Factor 3: Annual evaluation

No additional explanation required.

Factor 4: Evaluation of reports

For delegates that are NCQA Accredited in CR, the only NCQA-required reporting is the names or files of practitioners or providers processed by the delegate.

Factor 5: Annual audit of credentialing information integrity

If the organization delegates any credentialing activities covered in the scope of these standards, the organization or the delegate annually audits (as applicable) the delegate's credentialing files for inappropriate documentation and updates to:

- The application and attestation.
- Credentialing documents received from the source or agent.
- Documentation of completion of credentialing activities:
- Verification dates.
- Report dates.
- Credentialing decision dates.
- Signature or initials of the verifier or reviewer.
- Credentialing checklist, if used.

Inappropriate documentation and inappropriate updates. The following are inappropriate documentation and updates:

- Falsifying credentialing dates (e.g., licensure dates, credentialing decision dates, staff verifier dates, ongoing monitoring dates).
- Creating documents without performing the required activities.
- Fraudulently altering existing documents (e.g., credentialing minutes, clean-file reports, ongoing monitoring reports).
- Attributing verification or review to an individual who did not perform the activity.
- Updates to information by unauthorized individuals.

Note: *The organization may use the same file sample for factors 2 and 5.*

For each delegate, the audit universe includes practitioner files processed by the delegate for all initial credentialing decisions made and recredentialing decisions made or due to be made within the look-back period.

If the organization conducts the annual audit, it audits each delegate using one of the following methods:

- 5% or 50 files, whichever is less.
 - The sample includes at least 10 credentialing files and 10 recredentialing files. If fewer than 10 practitioners were credentialed or

recredentialed since the last annual audit, the organization audits the universe of files.

- The NCQA “8/30 methodology” available at <https://www.ncqa.org/programs/health-plans/policy-accreditation-and-certification/>

Either methodology is allowed, for consistency with other delegation oversight requirements for annual file audits.

A delegate that conducts the annual audit has two audit options.

- *Option 1:* Audit each client using one method:
 - The “5% or 50 files” method, *or*
 - The NCQA “8/30” method.
- *Option 2:* Conduct one audit across all clients if the delegate uses the same staff, policies and procedures and CR system for all clients. In this case:
 - The delegate must demonstrate that the same staff, policies and procedures and CR system are used for all clients.
 - The audit universe includes practitioner files processed for its clients by the delegate for all initial credentialing decisions made and recredentialing decisions made or due to be made within the look-back period.
 - The delegate must audit using the “5% or 50 files” method.

The organization or delegate may choose to audit more practitioner files than NCQA specifies.

The organization provides an auditing and analysis report for each delegate that includes:

- The date of the report.
- The title of staff who conducted the audit.
- The audit methodology:
- “5% or 50 files” or 8/30 methodology, as applicable.
- The audit period.
- The audit universe size (Audit universe is described above).
- The audit sample size.
- The file identifier (individual practitioner).
- The type of credentialing information audited (e.g., licensure).
- Findings for each file.
- The Conclusion, in the event of inappropriate documentation and updates occur (Element A, factor 2).
- Number or percentage and total inappropriate documentation and updates by type of credentialing information.

The delegate or organization must provide a completed audit report even if there are no inappropriate findings.

If the organization uses the delegate's audit results, it must provide evidence (e.g., report, meeting minutes) that it reviewed and evaluated the delegate's findings.

Factor 6: Implement corrective actions

For each delegate with inappropriate documentation and updates (findings) identified in factor 5, the organization or delegate documents corrective actions taken or planned, including the time frame for actions, to address all findings identified in factor 5. One action may be used to address more than one finding, if appropriate.

The organization's corrective action plan identifies staff (by title) who are responsible for implementing corrective actions.

The organization reviews (e.g., report, meeting minutes) and approves corrective action plans that are developed and implemented by its delegate.

Factor 7: Measure effectiveness of actions audit

The organization or delegate audits the effectiveness of corrective actions (factor 6) on findings for each delegate within 3–6 months of the annual audit completed for factor 5.

For each delegate, the audit universe includes practitioner files processed by the delegate for all initial credentialing decisions made and for recredentialing decisions made or due to be made 3–6 months after the annual audit.

The organization or delegate conducts a qualitative analysis if it identifies integrity issues during the follow-up audit.

If the organization uses the delegate's audit results, the organization must provide evidence (e.g., a report, meeting minutes, other evidence) that it reviewed and evaluated the delegate's findings.

The organization draws conclusions on the overall effectiveness of corrections implemented.

Exceptions

The element is NA if:

- The organization does not delegate credentialing activities.
- Delegation arrangements have been in effect for less than 12 months.

Factor 2 is NA if no practitioners were initial credentialed or were due for recredentialing.

Factors 2–7 are NA for Interim Surveys.

Factors 6–7 are NA if the organization's annual audit of all delegates' credentialing files did not identify any inappropriate documentation and updates to credentialing information used in the credentialing process. This must be evident in reports reviewed for factor 5.

Factor 7 is NA if the timing of the organization's annual audit is less than 3 months before the organization's NCQA Survey.

Related information

Use of collaborative. The organization may collaborate in a statewide, annual file audit and evaluation with other organizations that have overlapping practitioner and provider networks. The organizations in the collaborative use the same audit tool and share data.

Auditing NCQA Certified CR and CVO delegates. The organization is not required to audit NCQA-Certified CR or CVO delegates against timeliness requirements during the delegation audit, because NCQA does not recognize these delegates for decision making. If the organization delegates decision making, NCQA assesses the organization for timeliness of the credentialing decision.

Oversight of national delegates. NCQA allows a national corporate office to perform credentialing oversight of a nationally contracted delegate on behalf of its affiliated organizations (accreditable entities). Oversight results must be available for each accreditable entity survey. The organization reviews 75 randomly selected files across all Accreditable entities.

If the delegate's credentialing system is not centralized, separate oversight audits must be conducted for each Accreditable entity.

Examples

Factor 2: Annual evaluation

- Site visit.
- Telephone consultation.
- Documentation review.
- Committee meetings.
- Virtual review.

Factor 5: Audit and analysis report

[Delegate Name]—Annual CR Information Integrity Assessment Report

Report date: January [date, current year].

Auditor: [Delegate's Staff Name], Credentialing Director, [Delegate Name]

Auditing methodology: Each January, [Delegate Name] credentialing director audits a random sample of initial and recredentialing files for inappropriate documentation and updates for the previous calendar year. The organization randomly samples and audits 5% or 50 files (whichever is less) of all credentialing decisions made or due in the previous year.

- Period reviewed: January 1–December 31 of the previous calendar year.
- File-review universe size: The organization initial credentialed 2,000 practitioners, and recredentialed 8,000 practitioners who were due for recredentialing in the previous year.
- Audit sample size calculation: 10,000 practitioner files x 0.05 = 500 files.
- Audit sample size: 50 files, which is less than the 5% (500 files). The organization audited a random sample of 50 files that included 20 initial credentialing files and 30 recredentialing files.

Audit date: January 6–9, [current year].

Audit log: Attachment I. Partial illustration in table below.

Practitioner ID	File Type (Initial/Recred)	Inappropriate Documentation/ Updates	Credential Affected	Finding
Practitioner 1	Recredential	No		NA
Practitioner 2	Initial	No		NA
Practitioner 3	Recredential	Yes	Attestation	Attestation date updated by [Staff Name] instead of practitioner because attestation was expiring. 3/4/[previous year] @ 2:59 PM
Practitioner 4	Recredential	Yes	Licensure Sanction Information	Verification of licensure and sanction information updated by [Staff Name] without going to the source (3/4/[previous year] @ 11:00 AM) because the committee meeting was scheduled for the next day.

Summary of findings:

Credentialing Information Reviewed	Noncompliant Initial Credentialing Files	Noncompliant Recredentialing Files	Percentage of Noncompliant Modifications
Application and Attestation	4	4	16%
License	2	2	8%
DEA/CDS	0	0	0%
Education and Training	0	NA	0%
Board Certification Status	0	0	0%
Work History	0	NA	0%
Malpractice History	0	0	0%
Sanction Information	2	2	8%
Credentialing Committee Minutes	0	0	0%
Clean-File Approvals	0	0	0%
Ongoing Monitoring Reports	0	0	0%

Qualitative analysis: The credentialing director provided the credentialing staff with the audit log documenting how, when and by whom files were updated.

[Delegate Name]'s credentialing director held a series of meetings (January 14–17, [current year]) with credentialing staff (credentialing assistant director, credentialing manager, credentialing analyst) to determine the causes of each inappropriate update. The causes of the inappropriate updates are outlined in the table below.

Credentialing Information	Description of Noncompliant Update	Reason
Application and Attestation	Attestation date updated by staff instead of practitioner.	Staff spoke with the practitioner, who stated that all information remained accurate. Staff did not know that only the practitioner can update the information.
License	Verification was updated without going to the source.	Staff responsible for verification of licensure and sanction information was on emergency leave and did not complete the verification. Because temporary staff did not have time to complete verification of all practitioners, they copied existing credentials, changed dates and uploaded the information into the CR system before the Credentialing Committee meeting.
Sanction Information	Verification was updated without going to the source.	

Factor 6: Corrective actions

[Delegate Name]'s Credentialing Director shared the audit analysis results and mitigation recommendations with [Organization Name]'s Credentialing Director on January 31, [current year]. [Organization Name]'s Credentialing Director and the organization's leadership reviewed the report provided by [Delegate Name] (minutes attached) and required [Delegate Name] to implement immediate corrective actions and complete all corrective actions on or before the dates outlined in the table below.

Credentialing Information/ Noncompliant Update	Reason	Correction Actions Planned
Application and Attestation: Attestation date updated by staff instead of by practitioner.	Staff spoke with the practitioner, who stated that all information remained accurate. Staff did not know that only the practitioner can update the information.	[Delegate Name] will educate [Delegate Name]'s staff on the organization's policies and procedures by 4/15/[current year]. Owner: CR Director, [Delegate Name] [Delegate Name] will train [Delegate Name]'s staff on NCQA's documentation requirements by 4/15/[current year]. Owner: CR Director, [Delegate Name] [Delegate Name] will establish automated resending of attestation to practitioner 60 days before expiration by 4/15/[current year]. Owner: CR Director, [Delegate Name]
License: Verification was not updated from the source.	Staff responsible for verification of licensure and sanction information was on emergency leave and did not complete verification.	[Delegate Name] will require [Delegate Name]'s credentialing staff to undergo ethics training, with emphasis on following the organization's processes even if under pressure to take shortcuts by 5/1/[current year]. Owner: CR Director, [Delegate Name]
Sanction Information: Verification was not updated from the source.	Because temporary staff did not have time to complete verification of all practitioners, they copied existing credentials, changed dates and uploaded the information into the CR system before the Credentialing Committee meeting.	[Delegate Name] will incorporate system flags that do not allow updating information without going to the source and require confirmation that the information was received from the source by 6/1/[current year]. Owner: Information System Director, [Delegate Name] [Delegate Name] will purchase software application to automatically retrieve verification from accepted sources (web crawler) by 6/1/[current year]. Owner: Information System Director, [Delegate Name]

Evidence of [Organization Name]’s review: Minutes attached (Not shown in example.)

Factor 7: Effectiveness of corrective actions

[Delegate Name]—Annual CR Information Integrity Measure of Effectiveness

Report date: August 30, [current year].

Auditor: [Delegate Staff’s Name], Credentialing Director, [Delegate Name]

Auditing methodology: [Delegate’s Name] audits the effectiveness of corrective actions taken 6 months of the annual audit completed on January [date, year], by randomly selecting a sample of 5% or 50 files (whichever is less) initial and recredentialing files for inappropriate documentation and updates made by the [Delegate Name] since the last annual audit.

- Period reviewed: February [current year]–July [current year].
- File-review universe size: 5,000 files.
- Audit sample size calculation: 5,000 practitioner files x 0.05 = 250 files.
- Audit sample size: 50 files, which is less than the 5% (250 files). The organization reviewed a random sample of 20 initial credentialing files and 30 recredentialing files.

Audit date: July 11–15, [current year].

Audit log: Attachment I. (Not shown in this example.)

Summary of findings:

Credentialing Information Reviewed	Noncompliant Initial Credentialing Files	Noncompliant Recredentialing Files	Percentage of Noncompliant Modifications
Application and Attestation	0	0	0%
License	0	0	0%
Sanction Information	0	0	0%

Qualitative analysis: Not required.

Actions’ effectiveness:

Noncompliant Credentialing Updates	Corrective Actions Completed	Action Effectiveness Audit
Application and Attestation: Attestation date updated by staff instead of by practitioner.	[Delegate Name] educated staff on the organization’s policies and procedures. [Date] [Delegate Name] trained staff on NCQA documentation requirements. [Feb] [Delegate Name] established automated resending of attestation to practitioner 60 days before expiration. [Mar]	These actions have eliminated updating of attestation by staff. There were no incidences identified in the audit.

Noncompliant Credentialing Updates	Corrective Actions Completed	Action Effectiveness Audit
License: Verification was not updated from the source.	[Delegate Name] required credentialing staff to undergo ethics training, with emphasis on following the organization's processes even if under pressure to take shortcuts. [Feb] [Delegate Name] incorporated system flags that do not allow updating information without going to the source and require confirmation that the information was received from the source. [Mar] [Delegate Name] purchased software application to automatically retrieve verification from accepted sources (web crawler). [Apr]	The were no incidences identified in the audit.
Sanction Information: Verification was not updated from the source.		

Overall effectiveness—Conclusion: [Delegate Name]'s Credentialing Director shared the follow-up audit analysis results with the [Organization Name]'s Credentialing Director on August 1, [current year]. [Organization Name]'s Credentialing Director and leadership reviewed the report provided by [Delegate Name] on August 15, [current year], which proved the corrective actions implemented have been effective in preventing inappropriate documentation and updates based on follow-up assessment and the fact that no incidences of inappropriate documentation and updates were made.

Element D: Opportunities for Improvement

For delegation arrangements that have been in effect for more than 12 months, at least once in each of the past 2 years, the organization identified and followed up on opportunities for improvement, if applicable.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization has acted on identified problems, if any, at least once in each of the past 2 years that the delegation arrangement has been in effect	The organization took inappropriate or weak action, or acted only in the past year	The organization has not acted on identified problems

Data source Documented process, Reports, Materials

Scope of review	Product lines <i>For First Surveys and Renewal Surveys: This element applies to all product lines.</i> Documentation NCQA reviews reports of opportunities for improvement, from up to four randomly selected delegates, or from all delegates, if the organization has fewer than four, and for evidence that the organization took appropriate action to resolve issues. <i>For First Surveys:</i> NCQA reviews the organization's most recent annual review and follow-up on improvement opportunities. <i>For Renewal Surveys:</i> NCQA reviews the organization's most recent and the previous year's annual reviews and follow-up on improvement opportunities. The score for the element is the average of the scores for all delegates.
Look-back period	<i>For First Surveys:</i> At least once during the prior year. <i>For Renewal Surveys:</i> 24 months.
Explanation	This element may not be delegated. This element does not apply to CR Information Integrity. Areas of improvement for information integrity are addressed in Element C, factors 5-7. NCQA-Accredited/Certified delegates Automatic credit is available for this element if all delegates are NCQA-Accredited health plans or MBHOs, NCQA Accredited in CR or NCQA-Certified CVOs, unless the element is NA. NCQA-Certified CVOs must be certified to perform the delegated activity. Note: <i>For organizations that have both NCQA-Accredited/Certified and non-Accredited/Certified delegates:</i> • <i>NCQA-Accredited/Certified delegates are eligible for automatic credit.</i> • <i>Non-Accredited/Certified delegates are reviewed and scored accordingly.</i>
	Identify and follow-up on opportunities The organization uses information from its predelegation evaluation, ongoing reports, or annual evaluation to identify areas of improvement.
	Exceptions This element is NA if: • The organization does not delegate credentialing activities. • Delegation arrangements have been in effect for less than 12 months. • The organization has no opportunities to improve performance. – NCQA evaluates whether this conclusion is reasonable, given assessment results. This element is NA for Interim Surveys.
Examples	None.